

PREMIER HEALTHCARE ALLIANCE, L.P.
Data Stewardship Program

PART V

**PROTECTED HEALTH INFORMATION
POLICIES**

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PH-1. Policies Regarding Contracting with Covered Entities, Business Associates, and Subcontractors

A. Contracting With Covered Entities.

1. General Policy. Premier protects the privacy and security of Protected Health Information received from or on behalf of Covered Entities. Premier may only provide services to or on behalf of a Covered Entity involving the creation, receipt, maintenance, use, disclosure and/or transmission of Protected Health Information if it enters into a contract with the Covered Entity that contains the Business Associate Agreement provisions required by Section A.4 below. Premier should propose its template Business Associate Agreement provisions in addendum or free-standing agreement format to a Covered Entity. If a Covered Entity requests use of its own form of Business Associate Agreement provisions or requests revisions to Premier's template Business Associate Agreement provisions, Premier Personnel must refer such request to the appropriate Privacy Officer. The appropriate Privacy Officer may approve changes to the template provisions provided that the changes are consistent with this Policy.

2. Applicability of this Policy. This Policy does not apply to Premier if it acts as a Covered Entity. It only applies to arrangements where Premier Uses or Discloses Protected Health Information received from or on behalf of Covered Entities. (If Premier is a Covered Entity, see PH-1.B, below.) If Premier receives Protected Health Information from or on behalf of a Covered Entity, Premier and the Covered Entity must enter into a Business Associate Agreement. When Premier enters into an arrangement to act as a service provider, Premier Personnel must determine whether the customer is a Covered Entity, and if so, whether Premier will be serving as a Business Associate. Premier serves as a Business Associate whenever it will create, receive, maintain, transmit, Use, or Disclose Protected Health Information received from or on behalf of Covered Entities. The contracting process cannot be completed until these questions are answered and all necessary steps are undertaken.

3. Form of Business Associate Agreement. When possible, Premier will ask Covered Entities to sign Premier's form of Business Associate Agreement. If a Covered Entity refuses to sign Premier's form because the Covered Entity: (i) requests modifications to certain terms of the Business Associate Agreement; or (ii) requests Premier to use the Covered Entity's form, then Premier will review each request on a case-by-case basis. All requests for modification will be forwarded to the Premier Privacy Officer or his/her designee. Premier should ask Covered Entities to identify any Highly Confidential Information to be received by Premier under a Business Associate Agreement.

4. Required Elements and Provisions of Business Associate Agreement. If a contract is a Business Associate Agreement, it must be in writing and include the elements and provisions discussed below.

i. Permitted and Required Uses and Disclosures, Generally. The Business Associate Agreement must provide that Premier may only use or further disclose Protected Health Information as permitted or required by the Business Associate Agreement or as Required By Law. The Business Associate Agreement must specify and

describe the permitted and required uses and disclosures of Protected Health Information by Premier. Premier may not re-disclose Protected Health Information to third parties, including any Subcontractors, except as set forth in the Business Associate Agreement.

ii. Compliance with Privacy Standards. The Business Associate Agreement may not permit or require Premier to use or further disclose the Protected Health Information in a manner that would violate the obligations of the Covered Entity under the Privacy Standards, if done by the Covered Entity. To the extent that Premier carries out any of the Covered Entity's obligations under the Privacy Standards, the Business Associate Agreement must provide that Premier will comply with the requirements of Privacy Standards that apply to the Covered Entity in the performance of such obligations.

iii. Proper Management and Administration. Unless the appropriate Privacy Officer approves an exception, the Business Associate Agreement must permit Premier to:

(a) Use Protected Health Information for the proper management and administration of Premier or to carry out Premier's legal responsibilities;

(b) Disclosure if Required by Law; and

(c) Disclose Protected Health Information to a third person for the proper management and administration of Premier or to carry out Premier's legal responsibilities if Premier obtains reasonable assurances from (*e.g.*, in the form of a written confidentiality with) the person to whom the information is disclosed under which the person agrees to hold the Protected Health Information confidentially and only use or further disclosed the Protected Health Information as Required By Law or for the purpose for which the Protected Health Information was disclosed to the person.

iv. Data Aggregation Services, De-Identification and Limited Data Sets. The Business Associate Agreement:

(a) May permit Premier to Provide Data Aggregation services relating to the Health Care Operations of the Covered Entity and other Covered Entities;

(b) Unless the appropriate Privacy Officer approves an exception, must permit Premier to use Protected Health Information to create De-Identified Health Information in accordance with the Privacy Standards and disclose the De-Identified Health Information for purposes specified in the Business Associate Agreement or services contract; or

(c) Unless the appropriate Privacy Officer approves an exception, must permit Premier to use Protected Health Information to create a

Limited Data Set and disclose the Limited Data Set to a data user who has entered into a Data Use Agreement in accordance with these Policies.

v. Safeguards by Business Associate. The Business Associate Agreement must provide that Premier will use appropriate safeguards to prevent use or disclosure of Protected Health Information other than as provided for by the Business Associate Agreement. In addition, it must provide that Premier will comply with the Security Standards to the extent that Premier will receive or have access to Electronic Protected Health Information.

vi. Mitigation. The Business Associate Agreement must require Premier to mitigate any harmful effects of any improper use or disclosure known to Business Associate.

vii. Minimum Necessary Standard. The Business Associate Agreement must require that each party request, use or disclose, as applicable, only the minimum amount of Protected Health Information necessary for the purpose of the request, use or disclosure unless an exception to the Privacy Standards' minimum necessary standard applies.

viii. Reporting by Business Associate. The Business Associate Agreement must provide that Premier will report to the Covered Entity:

(a) Any use or disclosure of Protected Health Information not provided for by the Business Associate Agreement of which Premier becomes aware;

(b) Any Security Incident without unreasonable delay if Premier will maintain Electronic Protected Health Information;

(c) Any Breach of Unsecured Protected Health Information without unreasonable delay and in any event within sixty (60) days of discovery of the Breach.

ix. Subcontractors. The Business Associate Agreement must provide that Premier will enter into a written agreement meeting the requirements of 45 C.F.R. §§ 164.504(e) and 164.314(a)(2) with each Subcontractor that creates, receives, maintains or transmits Protected Health Information on behalf of Premier. The Business Associate Agreement must require Premier to ensure that the written agreement with each Subcontractor obligates the Subcontractor to comply with restrictions and conditions that are at least as restrictive as the restrictions and conditions that apply to Premier under the terms of the Business Associate Agreement.

x. Inspection and Copying. The Business Associate Agreement must provide that Premier will make available Protected Health Information in a Designated Record Set to the Covered Entity in accordance with 45 C.F.R. § 164.524. The Business Associate Agreement must include the access provision even if Premier will not maintain

any Designated Records Sets, but may include an acknowledgement that Premier will not maintain a Designated Record Set pursuant to the Business Associate Agreement.

xii. Amendments to Designated Record Set. The Business Associate Agreement must provide that Premier will make Protected Health Information contained in the Designated Record Set available for amendment and incorporate any amendments into the Designated Record Set in accordance with 45 C.F.R. § 164.526. The Business Associate Agreement must include the amendment provision even if Premier will not maintain any Designated Records Sets, but may include an acknowledgement that Premier will not maintain a Designated Record Set pursuant to the Business Associate Agreement.

xiii. Accounting. The Business Associate Agreement must provide that Premier will make available the information required for the Covered Entity to provide an accounting of disclosures to individuals in accordance with the Administrative Simplification Provisions at 45 C.F.R. § 164.528.

xiv. Inspection by Secretary. The Business Associate Agreement must provide that Premier will make its internal practices, books, and records relating to the use and disclosure of Protected Health Information received from, or created or received by Premier available to the Director, Office for Civil Rights of HHS for purposes of determining the Covered Entity's compliance with the Administrative Simplification Provisions.

xv. Termination for Material Breach. The Business Associate Agreement must provide that the Covered Entity may terminate the entire Business Associate Agreement (and not only the Business Associate provisions) if the Covered Entity determines that Premier has materially breached a Business Associate provision.

xvi. Return of Protected Health Information; Post-Termination Obligations. The Business Associate Agreement must provide that at termination of the Business Associate Agreement, if feasible, Premier will return or destroy all Protected Health Information received from, or created or received by Premier on behalf of the Covered Entity, that Premier still maintains in any form and retain no copies of such information or, if such return or destruction is not feasible, extend the protections of the Business Associate Agreement to the information and limit further uses and disclosures to those purposes that make the return or destruction of the information infeasible.

xvii. State Law Requirements. The Business Associate Agreement must comply with other requirements imposed by applicable state laws that are more stringent than HIPAA.

5. Non-Compliance with Business Associate Agreement. If a member of the Personnel knows of a pattern of activity or a practice of the Covered Entity that constitutes a violation of the Business Associate Agreement, the Personnel member must promptly notify the appropriate Privacy Officer. The appropriate Privacy Officer will investigate the manner, in

consultation with the General Counsel, consistent with these Policies, and if it determines that a violation of the Business Associate Agreement has occurred or is occurring, the appropriate Privacy Officer will ensure that Premier takes steps to cure the breach of the Business Associate Agreement or end the violation and notify the Covered Entity. If the steps are unsuccessful, Premier will terminate the Business Associate Agreement and return or destroy all Protected Health Information received from, or created or received by Premier on behalf of, the Covered Entity that Premier still maintains in any form and retain no copies of such information or, if such return or destruction is not feasible, extend the protections of the Business Associate Agreement to the information and limit further Uses and Disclosures to those purposes that make the return or destruction of the information infeasible.

6. Audits by Covered Entities.

i. General Procedure. Premier responds to Covered Entity requests to perform audits of Premier's compliance with Applicable Law governing the Use and Disclosure of Protected Health Information and compliance with its Business Associate Agreements. Premier also audits Subcontractors in accordance with this Policy.

ii. Audit Requests. Premier will accommodate a reasonable request by a Covered Entity to audit Premier's policies, procedures, and records relevant to the Business Associate Agreement for compliance with Applicable Law or the Business Associate Agreement, upon approval by the appropriate Privacy Officer. Audit requests will be processed subject to the following restrictions:

(a) The audit must not cause Premier to violate any contractual or other legal obligations or privileges;

(b) The Covered Entity and Premier are parties to a Business Associate Agreement and the audit is performed in accordance with the Business Associate Agreement;

(c) Premier receives reasonable advance written notice of the audit of no less than the number of days required by the Business Associate Agreement or ten (10) business days if the Business Associate Agreement does not specify a notice period;

(d) Premier will allow access to the minimum necessary Protected Health Information after consultation with the applicable Premier Privacy Officer and Premier legal counsel, as necessary and appropriate;

(e) The Covered Entity will only be allowed on the premises for purposes of inspection during normal business hours of Premier; and

(f) In no case will any books, papers, records or any other items belonging to Premier be removed from Premier premises without the prior written authorization of the applicable Premier Privacy Officer.

(g) Premier may request access to a Subcontractor's policies, procedures, books and records in accordance with the Downstream Subcontractor Agreement between the parties.

7. Covered Entity Notice of Privacy Practices.

i. General Policy. If required by the Business Associate Agreement with a Covered Entity, Premier will comply with applicable provisions of the Covered Entity's Notice of Privacy Practices.

ii. Compliance with Notice of Privacy Practices. Covered Entities are required to maintain a Notice of Privacy Practices but Business Associates are not. Where Premier is a Business Associate, Premier will comply with a Covered Entity's Notice of Privacy Practices to the extent applicable to Premier's activities and required by the Business Associate Agreement.

iii. Request for Notice of Privacy Practices. If a Business Associate Agreement requires Premier to comply with the Covered Entity's Notice of Privacy Practices, Premier will request a copy of the current Notice from the Covered Entity or locate it on the Covered Entity's website.

B. Contracting with Business Associates.

1. General Policy. Premier, in its Covered Entity capacity, ensures that its Business Associates protect patients' right to privacy consistent with Premier's obligations under federal and state law and Premier's privacy policies. Further, Premier may only enter into a Business Associate Agreement if it contains the Business Associate Agreement provisions required by Section B.3 below.

2. Applicability of this Policy. The first step under this Policy is to determine whether this Policy applies to the proposed arrangement or contract. This Policy does not apply to every arrangement or contract between Premier and another person or organization. It only applies where Premier functions as a Covered Entity and to a contract with a person or organization that meets the definition of a Business Associate. If the proposed contract is with a Business Associate, then it is a Business Associate Agreement and subject to this Policy. A Business Associate Agreement is not required with persons or organizations whose functions, activities or services do not involve the use or disclosure of Protected Health Information, and where any access to such Protected Health Information would be de minimis, if at all. For instance, Premier would not be required to enter into a Business Associate Agreement with its cleaning service because the performance of such services does not involve the use or disclosure of Protected Health Information, except incidentally, provided that reasonable safeguards are in place.

3. Form of Business Associate Agreement. When possible, Premier will ask the Business Associate to sign Premier's form of Business Associate Agreement. If a Business Associate refuses to sign Premier's form because the Business Associate: (i) requests

modifications to certain terms of the Business Associate Agreement, or (ii) requests Premier to use the Business Associate's form, then Premier will review each request on a case-by-case basis. All requests for modification will be forwarded to the appropriate Privacy Officer or his/her designee.

4. Required Elements and Provisions of Business Associate Agreement. If a contract is a Business Associate Agreement, it must be in writing and include the elements and provisions discussed below. Premier should propose Premier's template Business Associate Agreement provisions in addendum or free-standing agreement format, as appropriate. If a Business Associate requests use of its own form of Business Associate Agreement provisions or requests revisions to Premier's template Business Associate Agreement provisions, Personnel must refer the request to the appropriate Privacy Officer.

i. Permitted and Required Uses and Disclosures. The Business Associate Agreement must provide that the Business Associate may only use or further disclose Protected Health Information as permitted or required by the Business Associate Agreement or as Required By Law. The Business Associate Agreement must specify and describe the permitted and required uses and disclosures of Protected Health Information by the Business Associate. The Business Associate Agreement may not permit or require the Business Associate to use or further disclose the Protected Health Information in a manner that would violate the obligations of Premier under the Administrative Simplification Provisions, if done by Premier. The Business Associate may not re-disclose Protected Health Information to third parties, including any Subcontractors, except as set forth in the Business Associate Agreement. The Business Associate Agreement may permit the Business Associate to:

(a) Use Protected Health Information for the proper management and administration of the Business Associate or to carry out the Business Associate's legal responsibilities;

(b) Disclose Protected Health Information to a third person for the proper management and administration of the Business Associate or to carry out the Business Associate's legal responsibilities if the disclosure is (i) Required By Law or (ii) Business Associate enters into a written agreement with the person to whom the information is disclosed under which the person agrees to hold the Protected Health Information confidentially and use or further disclosed the Protected Health Information only as Required By Law or for the purpose for which the Protected Health Information was disclosed to the person;

(c) Provide Data Aggregation services relating to the Health Care Operations of Premier and other Covered Entities if approved by the appropriate Privacy Officer;

(d) Use Protected Health Information to create De-Identified Health Information in accordance with the Privacy Standards and disclose the De-Identified Health Information for purposes specified in the Business Associate

Agreement if approved by the appropriate Privacy Officer; or Use Protected Health Information to create a Limited Data Set and disclose the Limited Data Set to a Data User who has entered into a Data Use Agreement in accordance with these Policies if approved by the appropriate Privacy Officer.

ii. Compliance with Privacy Standards. To the extent that the Business Associate carries out any of Premier's obligations under the Privacy Standards, the Business Associate Agreement must provide that the Business Associate will comply with the requirements of Privacy Standards that apply to Premier in the performance of such obligations.

iii. Safeguards by Business Associate. The Business Associate Agreement must provide that the Business Associate will use appropriate safeguards to prevent use or disclosure of Protected Health Information other than as provided for by the Business Associate Agreement. In addition, it must provide that the Business Associate will comply with the Security Standards to the extent that Business Associate will receive or have access to Electronic Protected Health Information.

iv. Mitigation. The Business Associate Agreement must require that the Business Associate to mitigate any harmful effects of any improper use or disclosure known to Business Associate.

v. Minimum Necessary Standard. The Business Associate Agreement must require that the Business Associate request, use or disclose only the minimum amount of Protected Health Information necessary for the purpose of the request, use or disclosure unless an exception to the Privacy Standards' minimum necessary standard applies.

vi. Reporting by Business Associate. The Business Associate Agreement must provide that the Business Associate will report to Premier:

(a) Any use or disclosure of Protected Health Information not provided for by the Business Associate Agreement of which the Business Associate becomes aware;

(b) Any Security Incident without unreasonable delay if the Business Associate will maintain Electronic Protected Health Information;

(c) Any Breach of Unsecured Protected Health Information without unreasonable delay and in any event within 60 days of discovery of the Breach. Notwithstanding the 60-day time period permitted by the Breach Notification Standards, Premier's template Business Associate Agreement provisions require reporting within five business days of discover. Any reporting period longer than five business days must be approved by the appropriate Privacy Officer.

vii. Subcontractors. The Business Associate Agreement must provide that the Business Associate will enter into a written agreement meeting the requirements of 45 C.F.R. §§ 164.504(e) and 164.314(a)(2) with each Subcontractor that creates, receives, maintains or transmits Protected Health Information on behalf of the Business Associate. The Business Associate Agreement must require the Business Associate to ensure that the written agreement with each Subcontractor obligates the Subcontractor to comply with restrictions and conditions that are at least as restrictive as the restrictions and conditions that apply to Business Associate under the terms of the Business Associate Agreement.

viii. Inspection and Copying. The Business Associate Agreement must provide that the Business Associate will make medical records and other Protected Health Information in a Designated Record Set available within fewer than 30 days so that Premier can respond to the individual in a timely fashion. Premier does not need to provide a patient with access to Protected Health Information held by a Business Associate if the only information the Business Associate holds is a duplicate of what Premier maintains. In addition, some Business Associates will not maintain any Designated Records Sets. The Business Associate Agreement must still include the access provision, but may include an acknowledgement that the Business Associate will not maintain a Designated Record Set pursuant to the Business Associate Agreement.

ix. Amendments to Designated Record Set. The Business Associate Agreement must provide that the Business Associate will make Protected Health Information contained in the Designated Record Set available for amendment and incorporate any amendments into the Designated Record Set in accordance with Premier's policies on amendment to the Designated Record Set within 30 or fewer days. Note that some Business Associates will not maintain any Designated Records Sets or will only maintain a copy of one or more of Premier's Designated Record Sets. The Business Associate Agreement must still include the amendment provision, but may include an acknowledgement that the Business Associate will not maintain a Designated Record Set pursuant to the Business Associate Agreement.

x. Accounting. The Business Associate Agreement must provide that the Business Associate will make available the information required for Premier to provide an accounting of disclosures to individuals in accordance with Administrative Simplification Provisions at 45 C.F.R. § 164.528 within 30 or fewer days.

xi. Inspection by Secretary. The Business Associate Agreement must provide that the Business Associate will make its internal practices, books, and records relating to the use and disclosure of Protected Health Information received from, or created or received by the Business Associate on behalf of Premier available to the Director, Office for Civil Rights of HHS for purposes of determining Premier's compliance with the Administrative Simplification Provisions.

xii. Termination for Material Breach. The Business Associate Agreement must provide that Premier may terminate the entire Business Associate

Agreement (and not only the Business Associate provisions) if Premier determines that the Business Associate has materially breached a Business Associate provision.

xiii. Return of Protected Health Information; Post-Termination Obligations. The Business Associate Agreement must provide that at termination of the Business Associate Agreement, if feasible, the Business Associate will return or destroy all Protected Health Information received from, or created or received by the Business Associate on behalf of, Premier that the Business Associate still maintains in any form and retain no copies of such information or, if such return or destruction is not feasible, extend the protections of the Business Associate Agreement to the information and limit further uses and disclosures to those purposes that make the return or destruction of the information infeasible.

xiv. State Law Requirements. The Business Associate Agreement must comply with other requirements imposed by applicable state laws that are more stringent than HIPAA.

5. Optional Provisions of Business Associate Agreement. If a contract is a Business Associate Agreement, Premier should consider including the following provisions:

i. An indemnification provision that requires Business Associate to reimburse Premier for any losses resulting from Business Associate's breach of the Business Associate Agreement provisions or a Breach of Unsecured Protected Health Information.

ii. A requirement that the Business Associate maintain insurance to protect against losses arising out of violations of the Privacy Standards;

iii. A requirement that the Business Associate comply with any reasonable limitation in Premier's Notice of Privacy Practices if the limitation could affect the Business Associate's use or disclosure of Protected Health Information or a reasonable restriction on the use or disclosure of Protected Health Information that Premier has agreed to or is required to abide by in accordance with these Policies, if the limitation could affect the Business Associate's use or disclosure of Protected Health Information.

iv. If the Business Associate will conduct HIPAA-standardized transactions, it is strongly recommended that the Business Associate Agreement require that Business Associate not take any of the following actions: (a) change the definition, Data Condition, or use of a Data Element or Segment in a Standard; (b) add any Data Elements or Segments to the maximum defined Data Set; (c) use any code or Data Elements that are either marked "not used" in the Standard's Implementation Specification or are not in the Standard's Implementation Specification(s); or (d) change the meaning or intent of the Standard's Implementation Specification(s).

v. The Business Associate Agreement may include a provision that automatically amends the Business Associate Agreement provisions to include future amendments to the Administrative Simplification Provisions.

6. Non-Compliance with Business Associate Agreement. If Premier learns of a pattern of activity or practice of a Business Associate that constitutes a material breach or violation of the Business Associate's obligations under the Business Associate Agreement, Premier should first take reasonable steps to cure the breach or end the violation, as applicable, and, if such steps are unsuccessful, then Premier will, if feasible, terminate the contract or arrangement.

C. Contracting With Subcontractors.

1. General Policy. Premier contractually obligates Premier's Subcontractors to protect the privacy and security of Protected Health Information received from or on behalf of a Covered Entity pursuant to a Business Associate Agreement. Any Subcontractor that has access to such Protected Health Information when performing a service that Premier has agreed to perform for, or on behalf of, a Covered Entity, must be subject to appropriate terms regarding the protection of the privacy and security of the information. In most cases, the Subcontractor will be subject to the same terms that apply to Premier pursuant to the Business Associate Agreement between Premier and such Covered Entity, but pursuant to a Subcontractor agreement between Premier and the Subcontractor (a "Downstream Subcontractor Agreement"). In rare cases, the Subcontractor will be subject to the terms and conditions set forth in a confidentiality agreement.

2. Applicability of this Policy to a Subcontract Relationship. This Policy does not apply to every arrangement or contract between Premier and a subcontractor. Rather, it only applies to an arrangement where the Subcontractor is performing a service for Premier in its capacity as a Business Associate, involving Protected Health Information of a Covered Entity.

3. Form of Downstream Subcontractor Agreement. When possible, Premier will ask Subcontractors who will be assisting Premier in its Business Associate capacity to sign Premier's form of Downstream Subcontractor Agreement. If a Subcontractor refuses to sign Premier's form because the Subcontractor: (1) requests modifications to certain terms of the Downstream Subcontractor Agreement, or (2) requests Premier to use the Subcontractor's form, then Premier will review each request on a case-by-case basis. All requests for modification will be forwarded to the Premier Privacy Officer or his/her designee.

4. Subcontractor Performing a Service that Premier has Agreed to Perform for a Covered Entity. The Downstream Subcontractor Agreement, detailing services that a Subcontractor will provide to Premier in connection with the engagement that Premier has agreed to perform for a Covered Entity, must include the provisions contained in the Business Associate Agreement between Premier and the applicable Covered Entity. The Downstream Subcontractor Agreement must list the permitted Uses and Disclosures of the Protected Health Information, and it may not permit the Subcontractor to Use or Disclose the Protected Health Information for any purpose other than that which Premier is permitted to Use and/or Disclose

the Protected Health Information under the Business Associate Agreement. In implementing the Downstream Subcontractor Agreement, the individual negotiating the Downstream Subcontractor Agreement on behalf of Premier should ensure that any timeframes established under the Downstream Subcontractor Agreement for purposes of the Subcontractor reporting or responding to Premier are shorter than Premier's corresponding obligations to Covered Entities under applicable Business Associate Agreement(s) or at least mirror what Premier has agreed with Covered Entities under applicable Business Associate Agreement(s).

5. Subcontractor Performing a Service for Premier's Proper Management and Administration. If Premier is disclosing Protected Health Information to a Subcontractor or other third party for Premier's own proper management and administration, the appropriate Privacy Officer may authorize the use of a confidentiality agreement instead of a Downstream Subcontractor Agreement, provided that (a) the applicable Business Associate Agreement does not require a fuller Downstream Subcontractor Agreement that flows down all of the Business Associate Agreement provisions and (b) the third party agrees to (i) keep the information confidential, (ii) only Use or further Disclose the information as Required by Law or for the purpose for which Premier Disclosed it to the party, and (iii) notify Premier immediately of any instances of which the party is aware in which the confidentiality of the information has been breached. A confidentiality agreement should be used rarely. Under certain circumstances, Premier may also require that the Subcontractor obtain Premier's prior written permissions before making such disclosures.

6. Minimum Necessary. Premier will limit the Subcontractor's or other third party's access, and will require in its Downstream Subcontractor Agreement that the Subcontractor limit the Protected Health Information it requests, to the Protected Health Information to the minimum necessary to enable the Subcontractor to perform its service on behalf of Premier.

7. Termination of Subcontract. Upon termination of a Downstream Subcontractor Agreement, the Premier Privacy Officer will take the following steps:

- i. Identify the type of information held by the Subcontractor, including number or records or other characteristics that identify the information.
- ii. Review the terms of: (i) the underlying Business Associate Agreement, to determine Premier's ongoing obligations with regard to the Protected Health Information, and (ii) the Downstream Subcontractor Agreement being terminated to determine whether the Subcontractor has agreed to return or destroy the Protected Health Information or whether the Subcontractor has the option to find that it is not feasible to return or destroy the Protected Health Information.
- iii. Work with the Subcontractor to facilitate the return or destruction of the Protected Health Information in compliance with the underlying Business Associate Agreement and the Downstream Subcontractor Agreement and in accordance with the following procedures:

(a) If the Protected Health Information is to be returned to Premier by the Subcontractor, the appropriate Privacy Officer will work with the Subcontractor to arrange for a secure return of the Protected Health Information. The appropriate Privacy Officer will review the returned information to ascertain whether the full scope of information was returned. The appropriate Privacy Officer will request the Subcontractor to sign a statement indicating that all Protected Health Information created or obtained through the Downstream Subcontractor Agreement was returned. The Premier Privacy Officer will seek to facilitate storage of the information that will permit Premier to meet its ongoing obligations under the Business Associate Agreement.

(b) If the Protected Health Information is to be destroyed by the Subcontractor, the appropriate Privacy Officer will obtain a description from the Subcontractor of all of the Protected Health Information that was destroyed including service dates, number of files or other identifying characteristics. The appropriate Privacy Officer will request the Subcontractor to sign a statement indicating that all Protected Health Information created or obtained through the Downstream Subcontractor Agreement was destroyed.

PH-2. Policies Regarding Use and Disclosure of Protected Health Information Generally

A. Applicability. The Policies contained in this PH-2 (Policies Regarding Use and Disclosure of Protected Health Information Generally), apply where Premier is a Covered Entity or a Business Associate.

B. Minimum Necessary Requirements.

1. Minimum Necessary Standard Generally. Subject to certain exceptions set forth in these Policies, when using or disclosing Protected Health Information, Premier will make reasonable efforts to limit the Use or Disclosure to the minimum amount of Protected Health Information necessary to accomplish the intended purpose of the Use or Disclosure.

2. Applicability of Minimum Necessary Standard. Premier will Use, Disclose or request the minimum necessary amount of Protected Health Information in all situations, unless an exception applies. As a general rule, Premier may not Use, Disclose or request the entire medical record or the name, Social Security Number and other Direct Identifiers of an individual unless the Direct Identifiers are specifically justified in writing as reasonably necessary to accomplish the purpose of the Use, Disclosure or request.

3. Exceptions. The minimum necessary requirements do not apply for Uses and Disclosures:

i. For Treatment. Disclosures to or requests by a Health Care Provider for Treatment except Disclosures of:

(a) Alcohol and Drug Abuse Treatment Program records or information, which are subject to additional Disclosure limitations under Applicable Law and these Policies; and

(b) Highly Confidential Information that is subject to minimum necessary requirements under Applicable Law other than HIPAA.

ii. To the Patient or Beneficiary. Uses or Disclosures made to the patient or beneficiary by Premier as a Covered Entity or by Premier as a Business Associate at the request of the Covered Entity.

iii. Pursuant to an Authorization. Uses or Disclosures pursuant to a valid Authorization (including, without limitation Authorizations to Use and Disclose Protected Health Information for Marketing purposes), in which case the extent of the Disclosure must be consistent with any limitations imposed by the Authorization.

iv. To HHS. Disclosures to the Director, Office for Civil Rights of HHS for HIPAA compliance purposes.

v. Required By Law. Uses or Disclosures that are Required By Law to the extent Required By Law.

vi. Required for Compliance with HIPAA Administrative Simplification Provisions. Uses or Disclosures that are required for compliance with HIPAA.

4. General Procedures for Implementing Minimum Necessary Standards.

i. Persons with Access. Premier's Privacy Officers, in conjunction with the Data Stewardship Committee, will identify Personnel and other persons (or classes of persons) within Premier who need access to Protected Health Information to carry out their duties.

ii. Categories of Access. For each person (or classes of persons), Premier's Privacy Officers will identify the category (or categories) of Protected Health Information to which access is needed and any conditions appropriate to such access.

iii. Limitation of Use of Protected Health Information. Once Premier identifies persons within Premier who need access to the categories of Protected Health Information, Premier must make reasonable efforts to limit access of such persons only to their respective identified categories.

iv. Minimum Necessary Criteria Protocols – Routine. When a Disclosure or Use is of the type that occurs on a routine or recurring basis, Premier will implement a standard protocol for Disclosure or Use within Premier that limits the Protected Health Information Disclosed or requested to the amount reasonably necessary to achieve the purpose of the disclosure.

v. Minimum Necessary Criteria Protocols - Non-Routine. For any non-routine Disclosure or Use, the Premier Privacy Officers:

(a) must develop criteria to limit the PHI Disclosed or Used to the amount reasonably necessary to accomplish the purpose of the Disclosure or Use;

(b) train designated Personnel to review Personnel requests for Use or Disclosure of PHI on an individual basis in accordance with such criteria; and

(c) have Premier designated Personnel review requests for Use or Disclosure on an individual basis in accordance with such criteria.

vi. Documentation of Minimum Necessary Criteria Protocols. For both routine and non-routine Uses and Disclosures, the appropriate Privacy Officer will document the information the parameters for Use and Disclosure of Protected Health Information in specific protocols ("Minimum Necessary Criteria Protocols").

vii. Responding to Requests for Uses or Disclosures. Premier Personnel may rely on a requested Use or Disclosure as the minimum necessary for the stated purpose (if reliance is reasonable under the circumstances) in the following situations:

(a) When making Disclosures to public officials if the requesting official represents that the information requested is the minimum necessary for the stated purpose;

(b) If Premier is a Business Associate, when the information requested to be Used or Disclosed is made by the Covered Entity that provided the Protected Health Information;

(c) If Premier is a Covered Entity, when the information is requested by another Covered Entity;

(d) If Premier is a Covered Entity, when the information is requested by a health care professional (e.g., a pharmacist, physician, or nurse) who is a member of Premier's Personnel or is a Business Associate of Premier for the purpose of providing professional services to Premier, if the professional represents that the information requested is the minimum necessary for the stated purpose(s); and

(e) If Premier is a Covered Entity, when the information is requested for research purposes and the person requesting the information has provided documentation or representations.

C. Incidental Uses and Disclosures of Protected Health Information.

1. General. Premier reasonably safeguards Protected Health Information in order to limit incidental Uses or Disclosures of Protected Health Information. Premier Personnel are careful to avoid inadvertent Disclosures as much as is reasonably practicable and to use caution and common sense when handling Protected Health Information.

2. Definition. An incidental Use or Disclosure is a secondary Use or Disclosure that cannot reasonably be prevented, is limited in nature, and occurs as by-product of an otherwise permitted Use or Disclosure.

3. Requirements. A Use or Disclosure of Protected Health Information which is the result of, or incidental to, an otherwise permissible Use or Disclosure is consistent with Premier's policies where Premier has:

i. implemented reasonable administrative, technical and physical safeguards to limit incidental Uses and Disclosures; and

ii. complied with the minimum necessary requirements set forth in these Policies and in any applicable Minimum Necessary Criteria Protocols.

4. Accounting. Incidental Uses and Disclosures made by Premier in accordance with this Policy are not required to be part of an Accounting.

D. Mitigation of Unauthorized Uses and Disclosures.

1. General. Premier mitigates, to the extent practicable, any harmful effect that is known to Premier to have occurred as a result of a Use or Disclosure of Protected Health Information in violation of the requirements of the Privacy Standards or Security Standards, or these Policies relating to the Use or Disclosure of Protected Health Information, either by Premier or one of its Subcontractors.

2. Reporting to Privacy Officer. Information regarding any Unauthorized Use or Disclosure discovered by any employee of Premier will be reported promptly to the Premier Privacy Officer.

3. Mitigation Plan. The Premier Privacy Officer, with any incident response team organized by the Premier Privacy Officer, and in consultation with Premier legal counsel, will develop and implement a plan as soon as reasonably practicable to mitigate any known or reasonably anticipated harmful effects from any Unauthorized Use or Disclosure (the “Mitigation Plan”). The Mitigation Plan may include the following elements, as appropriate:

i. Identifying the source(s) of the Unauthorized Use or Disclosure of Protected Health Information and taking appropriate corrective action;

ii. Contacting the recipient of the information that was the subject of the Unauthorized Use or Disclosure of Protected Health Information and requesting that such recipient either destroy or return the information;

iii. Instructing such recipient to make no further Uses or Disclosures of such Protected Health Information; and

iv. Reviewing, and correcting where appropriate, any policy or procedure of Premier that directly caused or contributed to the Unauthorized Use or disclosure of Protected Health Information.

4. Notifications. The Premier Privacy Officer will notify the Covered Entity, Upstream Business Associate, affected individual(s), the media, and HHS, as applicable, regarding an Unauthorized Use or Disclosure in accordance with these Policies.

5. Accounting. Any accounting will include such Unauthorized Uses or Disclosures.

E. Uses and Disclosures of Psychotherapy Notes.

1. General. Premier recognizes that a patient’s Psychotherapy Notes are subject to heightened protection under Applicable Law. As a Business Associate, Premier does not Use or Disclose a patient’s Psychotherapy Notes unless expressly directed by the Covered

Entity. As a Covered Entity, Premier does not Use or Disclose a patient's Psychotherapy Notes unless it has obtained an Authorization from the patient, except in certain very limited circumstances set forth in the Privacy Standards. The Premier Privacy Officer will assess each request for a Use or Disclosure of Psychotherapy Notes to determine whether the request is valid.

2. Definition. Psychotherapy Notes are defined in the Privacy Standards.

3. Alcohol and Drug Abuse Treatment Program Materials. If Psychotherapy Notes are recorded about an individual who is a patient of an Alcohol and Drug Abuse Treatment Program, the Psychotherapy Notes are also Alcohol and Drug Abuse Treatment Program records subject to restrictions set forth in federal regulations located at 42 C.F.R. Part 2 and these Policies.

4. Authorization Requirement. In order to Use or Disclose Psychotherapy Notes about a patient, the Covered Entity (or Premier if Premier is the Covered Entity) must have obtained an Authorization from the patient, except in certain very limited circumstances set forth in the Privacy Standards.

5. With a Valid Authorization. If the Covered Entity (or Premier if Premier is the Covered Entity) has a valid Authorization for the requested Use or Disclosure, Premier will make the requested Use or Disclosure after receiving permission from the Premier Privacy Officer.

6. No Authorization. If the Covered Entity (or Premier if Premier is the Covered Entity) does not have an Authorization, the appropriate Privacy Officer will assess the requested Use or Disclosure and make a case-by-case determination, in consultation with Premier's legal counsel and the Data Stewardship Committee, as necessary, regarding whether the Use or Disclosure is appropriate. The appropriate Premier Privacy Officer will not authorize the requested Use or Disclosure unless it determines that the request meets all requirements of Applicable Law.

7. No Right of Access. If a patient requests to access, inspect or obtain a copy of his/her Psychotherapy Notes, Premier will refer such requests to the Covered Entity (or to the appropriate Privacy Officer if Premier is the Covered Entity). If Premier is a Business Associate, Premier will not permit a patient to access, inspect or obtain a copy of Psychotherapy Notes regarding the patient except at the Covered Entity's written direction.

F. Prohibition on Sale of Protected Health Information

1. General Policy. Premier does not disclose Protected Health Information in exchange for Financial Remuneration or other remuneration unless the individual who is the subject of the Protected Health Information has authorized the disclosure in accordance with PH-3.B or PH-4.B or an exception in Section F.3 below applies. "Sale of Protected Health Information" means a disclosure of Protected Health Information by a Covered Entity or a Business Associate where the Covered Entity or Business Associate directly or indirectly receives Remuneration from or on behalf of the recipient of the Protected Health Information in

exchange for the Protected Health Information. A Sale of Protected Health Information does not include a disclosure made in accordance with Sections F.3.i – ix below.

2. General Procedure. If Premier desires to enter into a relationship with a third party under which Premier (or Premier's Business Associate) directly or indirectly receives Financial Remuneration or any other remuneration in exchange for an individual's Protected Health Information, the appropriate Privacy Officer must determine whether (1) an exception set forth in Section F.3 applies or (2) Premier has obtained a valid Authorization from the individual. If the appropriate Privacy Officer determines that no exception is available and Premier does not have a valid authorization from the individual, Premier must either seek an authorization from the individual or decline to make the disclosure in exchange for remuneration.

3. Exceptions for when an Authorization is not Required. Premier may receive remuneration in exchange for an individual's Protected Health Information without first obtaining the individual's authorization if the appropriate Privacy Officer confirms that the exchange is:

- i. For public health activities (i) in accordance with PH-3.D-3 or (ii) as part of a Limited Data Set in accordance with PH-2.I;
- ii. For Research purposes
 - (a) in accordance with PH-4.I or as part of a Limited Data Set in accordance with PH-2.I; and
 - (b) the only remuneration received by Premier (or its Business Associate) is a reasonable cost-based fee to cover the cost to prepare and transmit the Protected Health Information for the Research purpose;
- iii. For Treatment and Payment in accordance with PH-4.C;
- iv. For the sale, transfer, merger or consolidation of all or part of Premier with another Covered Entity, or an entity that following such activity will become a Covered Entity and due diligence related to such activity;
- v. To or by a Business Associate for activities that the Business Associate undertakes on behalf of Premier in accordance with PH-1.B, and the only remuneration provided is by Premier to the Business Associate for the performance of such activities;
- vi. To provide an individual with a copy of Protected Health Information in a Designated Record Set in accordance with PH-4.L;
- vii. To provide an individual with an accounting of accountable disclosures in accordance with PH-4.N;
- viii. When Required By Law in accordance with PH-4.D; or

ix. For any other purpose permitted by and in accordance with the applicable requirements of the Privacy Standards, where the only remuneration received by Premier or Premier's Business Associate is a reasonable, cost-based fee to cover the cost to prepare and transmit the Protected Health Information for such purpose or a fee otherwise expressly permitted by other law.

G. Special Communication Requirements.

1. General Rule. Covered Entities are required to accommodate an individual's reasonable request to receive communications of his or her Protected Health Information by alternative means or at alternative locations (e.g., at a business address).

2. Premier as Business Associate. Where Premier is a Business Associate, in the unlikely event that a Covered Entity customer requests Premier to communicate directly with a patient or beneficiary, Premier will take reasonable measures to accommodate a Covered Entity's request to communicate with the individual by alternative means or alternative locations in accordance with the applicable commercial agreement and Business Associate Agreement and with these Policies. The Covered Entity's request must be provided to Premier in writing, and Premier will promptly respond to any such request.

3. Premier as Covered Entity. Where Premier is a Covered Entity, Premier will accommodate any reasonable request by a patient to receive communications of Protected Health Information from Premier by alternative means or at alternative locations, provided that the following conditions are satisfied:

- i. The patient's request must be provided to Premier in writing.
- ii. Premier may condition its accommodation of the patient's request on whether: (i) the patient has provided Premier with information as to how Payment, if applicable, for services rendered will be handled; and (ii) the patient has specified an alternative address or other method of contact.
- iii. Premier may not condition the honoring of the patient's request on receiving an explanation from the patient as to the reason for the request.

H. Verification of Identity and Authority.

1. Verification of Identity. Premier verifies the identity of a person requesting Protected Health Information ("Requestor") and the authority of any Requestor to have access to Protected Health Information. Prior to disclosing Protected Health Information to a Requestor, Premier will take reasonable steps to verify the identity of the Requestor and the authority of the Requestor to have access to such Protected Health Information. Unless a Requestor is already known to Premier at the time of the request, Premier must request a driver's license or other picture identification, or other reasonable proof of a Requestor's identity.

2. Documentation. When a Disclosure is conditioned on the Requestor's production of particular documentation, statements, or representations (such as Disclosures for

health oversight activities, judicial and administrative proceedings or law enforcement activities), Premier will require the Requestor to provide, such documentation, statements, or representations. If such reliance is reasonable under the circumstances, Premier may rely on documentation, statements, or representations that, on their face, meet the applicable requirements.

I. De-Identified Health Information and Limited Data Sets.

1. Authority to Create De-Identified and Limited Data Sets.

i. Premier as Business Associate. Where Premier is a Business Associate, Premier does not Use Protected Health Information to create De-Identified Health Information or a Limited Data Set, unless such creation is authorized by the Business Associate Agreement with the Covered Entity and created in accordance with the procedures set forth below. Premier's form of Business Associate Agreement permits Premier to create a data set with only De-Identified Health Information in accordance with the HIPAA de-identification standard and this Policy.

If Premier receives Protected Health Information pursuant to a Data Use Agreement or Confidentiality Agreement with a Covered Entity or other party, Premier may Use and Disclose the Protected Health Information pursuant to the Data Use Agreement or Confidentiality Agreement and this Policy.

ii. Premier as Covered Entity. Where Premier is a Covered Entity, Premier may use or disclose De-Identified Health Information or a Limited Data Set without obtaining a patient's authorization. Health Information is De-Identified Health Information only if Premier has followed one of the two de-identification methods set forth in Section 2 below. Health Information constitutes a Limited Data Set only if the Direct Identifiers have been removed from the data set in accordance with Section 3 below of this Policy.

Premier may use Protected Health Information about a patient to create De-Identified Health Information or a Limited Data Set without the patient's authorization consistent with PH-4.C. Premier may disclose Protected Health Information to a Business Associate to create De-Identified Health Information or a Limited Data Set without the patient's authorization consistent with PH-4.C and PH-1.B.

Premier must comply with PH-4.I and its other policies and procedures governing Research when creating De-Identified Health Information or Limited Data Sets for Research purposes.

2. Procedures for De-Identifying Protected Health Information.

i. Methods for De-identification of Health Information. Premier may determine that Health Information is De-Identified only if Premier complies with one of the following de-identification procedures:

(a) De-Identified under statistical methods in accordance with these Policies; or

(b) Removal of certain specific identifiers in accordance with these Policies.

ii. De-identification Under Statistical Methods. A person with appropriate knowledge of and experience with generally accepted statistical and scientific principles and methods for rendering information not individually identifiable: (a) applying such principles and methods determines that the risk is very small that the information could be Used, alone or in combination with other reasonably available information, by an anticipated recipient to identify an individual who is a subject of the information; and (b) documents the methods and results of the analysis to justify such determination. When de-identifying Protected Health Information through the use of an expert, Premier ensures that the expert has relevant professional experience and academic or other training, as well as actual experience using health information de-identification methodologies. Premier requests that the expert assess the expected change of computational capability and access to various data sources to determine an appropriate timeframe within which the health information will be considered reasonably protected from identification. At the end of such timeframe, Premier examines whether future releases of the data to the same recipient (*e.g.*, monthly reporting) should be subject to additional or different de-identification processes to maintain the very low risk of identification standard.

iii. Removal of Identifiers. Health Information is considered De-Identified with respect to a patient or other individual if all eighteen (18) of the following identifiers (including any parts or derivatives thereof) of the patient or beneficiary or relatives, employers, or household members of the patient or beneficiary are removed and Premier does not have any actual knowledge that the information could be Used alone or in combination with other information to identify a patient or beneficiary. For purposes of this section, *actual knowledge* means clear and direct knowledge that the remaining information could be used, either alone or in combination with other information, to identify a patient who is a subject of the information. When relying on the removal of the eighteen (18) identifiers to achieve de-identification, Premier takes special care to assure that any free text or other unstructured data fields do not contain stray identifiers (for example, a hand-written name on an x-ray scan) or information that could be used to re-identify the patient (*e.g.*, noteworthy professional or athletic roles or accomplishments). The eighteen (18) identifiers are:

(a) Names (For example, Health Information is not De-Identified if it includes of the patient or beneficiary or relatives, employers or household members of the patient);

(b) geographic subdivisions smaller than a state (*e.g.*, street address, city, county, precinct, zip code and their equivalent geocodes, except for the initial three digits of a zip code, if according to the currently available data

from the Bureau of the Census, the geographic unit formed by combining all zip codes with the same three initial digits contains more than 20,000 people. If such geographic units contain 20,000 people or less, then the initial three digits of the zip codes must be changed to 000 and thus treat them as a single geographic area.);

(c) all elements of dates, except year, directly related to an individual including birth date, admission date, discharge date, date of death; and for all ages over 89, all elements of date including year indicative of such age, except that such ages and elements may be aggregated into a single category of age 90 or older. Note, however, that for Research or other studies relating to young children or infants, this Policy does not prohibit age of an individual from being expressed in months, days or hours;

(d) telephone numbers;

(e) fax numbers;

(f) electronic-mail addresses;

(g) social security numbers;

(h) medical record numbers;

(i) Health Plan beneficiary numbers;

(j) account numbers;

(k) certificate/license numbers;

(l) vehicle identifiers and serial numbers, including license plate numbers;

(m) device identifiers and serial numbers;

(n) web universal resource locators (URLs);

(o) internet protocol (IP) address numbers;

(p) biometric identifiers including finger and voice prints;

(q) full face photographic images and any comparable images;
and

(r) any other unique identifying number, characteristic or code, except as otherwise permitted for re-identification purposes in Section 9.

iv. Re-identification of Health Information. Premier may assign a code or other means of record identification to allow information De-Identified under these Policies to be re-identified by Premier, provided that:

(a) The code or other means of record identification is not derived from or related to information about the patient or beneficiary and is not otherwise capable of being translated so as to identify the individual;

(b) Premier does not Use or Disclose the code or other means of record identification for any other purpose, and does not Disclose the mechanism for re-identification;

(c) A code or other means of record identification designed to enable coded or otherwise De-Identified information to be re-identified may not be Disclosed except as provided in these Policies; and

(d) De-Identified Health Information that has been re-identified may not be Disclosed or Used except as otherwise permitted under Premier's policies for Disclosure and Use of Protected Health Information.

v. Final Approval. Before Premier Discloses any De-Identified Health Information created in accordance with this Policy to a recipient of the data set, the Premier Privacy Officer, or his/her designee, must confirm that Premier does not have actual knowledge that the information in the data set could be used, alone or in combination with other information that is reasonably available to the intended recipient of the data set, to identify a particular individual who is the subject of De-Identified Health Information contained in the data set.

3. Procedures for the Creation of Limited Data Sets.

i. Method for Creating a Limited Data Set. A Limited Data Set is a collection of Protected Health Information and other data that excludes the following "Direct Identifiers" of individuals who are the subject of the Protected Health Information or a relative, employer or household member of the individual:

- (a) Names;
- (b) Postal address information, other than town or city, state and zip code;
- (c) Telephone numbers;
- (d) Fax numbers;
- (e) Electronic mail address;
- (f) Social security numbers;

- (g) Medical record numbers;
 - (h) Health Plan beneficiary numbers;
 - (i) Account numbers;
 - (j) Certificate/license numbers;
 - (k) Vehicle identifiers and serial numbers (including license plate numbers);
 - (l) Device identifiers and serial numbers;
 - (m) Web Universal Resource Locators (URLs);
 - (n) Internet Protocol (IP) address numbers;
 - (o) Biometric identifiers (including finger and voice prints);
- and
- (p) Full face photographic images and any comparable images.

4. Use or Disclosure of a Limited Data Set by Premier. Where Premier is a Business Associate, Premier may only Use a Limited Data Set or Disclose a Limited Data Set if the intended user of such Limited Data Set (including Premier) has entered into a Data Use Agreement which complies with HIPAA's Limited Data Set standards. Where Premier is a Covered Entity, Premier may only Disclose a Limited Data Set if the intended user of such Limited Data Set (including Premier) has entered into a Data Use Agreement which complies with HIPAA's Limited Data Set standards.

5. Valid Purpose. Premier may only Use or Disclose a Limited Data Set for Research, public health, or the Health Care Operations purposes of a Covered Entity (which needs to be the Covered Entity that provided the Protected Health Information generating the Limited Data Set).

J. Reporting Unauthorized Uses or Disclosures. If Premier becomes aware or suspects that: (1) Premier or any Personnel member has Used or made a Disclosure of Protected Health Information in violation of the Privacy Standards or its Business Associate Agreements, or (2) a Subcontractor Used or made a Disclosure of Protected Health Information in violation of the Privacy Standards or its Downstream Subcontractor Agreements with Premier, Premier will investigate the actual or suspected Unauthorized Use or Disclosure and take appropriate corrective and remedial action and make appropriate notifications in accordance with the steps outlined in Part VI, "Privacy Event Response Policies".

PH-3. Policies Regarding Use and Disclosure of Protected Health Information as a Business Associate

A. Applicability. The Policies contained in this PH-3 (Policies Regarding Use and Disclosure of Protected Health Information as a Business Associate) apply only to arrangements where Premier Uses or Discloses Protected Health Information received from or on behalf of Covered Entities.

B. Authorization for Use and Disclosure.

1. Generally. A Covered Entity must obtain an Authorization form that complies with the requirements of Applicable Law (an “Authorization”) prior to using or disclosing Protected Health Information unless expressly permitted to Use or Disclose Protected Health Information without an Authorization under Applicable Law. A Covered Entity must also obtain Authorization in a form that complies with the requirements of applicable state and federal laws other than HIPAA before using or disclosing Highly Confidential Information in a manner that requires an Authorization under such laws. Premier must report to the Covered Entity if it is aware of a Use or Disclosure of Protected Health Information that requires the Covered Entity to obtain an Authorization from its patients or beneficiaries but for which the Covered Entity has not yet obtained an Authorization. Any Use or Disclosure permitted by Applicable Law, regardless of whether an Authorization is also required, must also comply with any corporate ethics and standards governing Premier.

2. Use or Disclosure Without an Authorization.

i. Provided that Premier is permitted to do so by the Business Associate Agreement with the Covered Entity, Premier may Use and Disclose Protected Health Information on behalf of the Covered Entity without an Authorization for: (A) Treatment, Payment or Health Care Operations; (B) public policy purposes in compliance with these Policies; (C) other uses or disclosures of Protected Health Information permitted to be performed without an Authorization pursuant to the Privacy Standards or (D) incidental disclosures or any other rare circumstances approved in advance by the Covered Entity’s Privacy Officer. Any Uses and Disclosures must also comply with any other Applicable Laws.

ii. Premier may Use and Disclose Protected Health Information as Required By Law to satisfy its legal compliance obligations or for the proper administration of Premier without an Authorization consistent with these Policies.

iii. Premier may Use and Disclose Protected Health Information under other rare circumstances, which are in accordance with Applicable Law as determined by the Premier Privacy Officer, in consultation with the Data Stewardship Committee and Premier legal counsel, as appropriate, on a case-by-case basis.

3. Use or Disclosure With an Authorization.

i. If Premier wishes or needs to Use or Disclose Protected Health Information for any purpose other than listed above in PH-3.B.2, the appropriate Privacy Officer, in consultation with the Data Stewardship Committee and Premier Legal Counsel, as appropriate, will determine whether it is permitted to do so, consult with the Covered Entity, and if applicable, determine whether the Covered Entity is required to obtain an Authorization from its patients or beneficiaries (as applicable).

ii. If the Covered Entity requests that Premier Use or Disclose Protected Health Information in a manner that requires an Authorization, the appropriate Privacy Officer may request that the Covered Entity provide Premier with a copy of a valid Authorization. If the Authorization is valid, Premier will comply with the request from the Covered Entity. If the Authorization is not valid or if the Covered Entity has not obtained an Authorization, Premier will not comply with the request. The appropriate Privacy Officer, in consultation with Premier's legal counsel, will determine whether the Authorization is valid.

4. Unauthorized Uses and Disclosures. If Premier learns of a circumstance where the Covered Entity has Used or Disclosed Protected Health Information without an Authorization when one was required, it will promptly notify the Covered Entity and consult with Premier legal counsel to determine whether further action is necessary or advisable.

5. Documentation Requirements. Premier will retain all documentation concerning a Use or Disclosure that is made in accordance with an Authorization for a minimum of six (6) years.

C. Uses and Disclosures As Required By Law or for Proper Administration and Management.

1. Uses of Protected Health Information for Management and Administration. Premier may Use Protected Health Information internally for its own proper management and administrative services, subject to the applicable Business Associate Agreement.

2. Disclosure for Management and Administration. Premier may Disclose Protected Health Information to third parties for its own proper management and administration, provided, however, that Premier obtains reasonable assurances from the person to whom the information is to be Disclosed that it will be held confidentially and be Used or further Disclosed only as Required by Law or for the purpose for which it was Disclosed to the person, and the person is required to notify Premier of any instances of which the person is aware in which the confidentiality of the information may have been Used or Disclosed inappropriately.

3. Legal Compliance Activities. Premier may Use and Disclose Protected Health Information as Required By Law or to carry out its legal responsibilities.

D. Uses and Disclosures for Public Policy Purposes of Covered Entities.

1. Generally. Under certain circumstances, the Privacy Standards permit a Covered Entity to Use and Disclose Protected Health Information without first obtaining an Authorization, based on the public policy considerations identified below. Personnel of Premier must refer any Covered Entity request for Premier, in its capacity as a Business Associate to Use or Disclose Protected Health Information on behalf of the Covered Entity for a public policy purpose of the Covered Entity to the appropriate Privacy Officer for approval prior to making the Use or Disclosure.

2. Public Policy Categories. For each category of public policy exception below, Premier will consult and follow the policies, procedures and instructions of the Covered Entity, subject in each case to the approval of the appropriate Privacy Officer, which will be granted only after review and consultation with the Data Stewardship Committee and Premier legal counsel, as necessary:

i. Uses and Disclosures Required By Law, provided that Premier may Use and Disclose Protected Health Information for Premier's own legal compliance purposes in compliance with these Policies;

ii. Disclosures for public health activities (such as public health reporting, child abuse reporting, FDA reporting, notification of exposure to disease or condition, Disclosures to employers, Disclosures to a school about a student or prospective student);

iii. Disclosures to report victims of abuse, neglect or domestic violence (other than child abuse);

iv. Disclosures for health oversight activities;

v. Disclosures for judicial and administrative proceedings;

vi. Disclosures to law enforcement officials;

vii. Disclosures about decedents;

viii. Disclosures to facilitate organ and tissue procurement;

ix. Disclosures for health or safety;

x. Uses and Disclosures for specialized government functions (such as military and veterans activities, for national security and intelligence activities and for protective services for the President and others);

xi. Disclosures to correctional institutions about inmates or other individuals; and

- xii. Disclosures to comply with workers' compensation programs.

E. Accounting of Non-Routine Disclosures of Protected Health Information.

1. Requirement. A Covered Entity is required to provide its patients or beneficiaries (as applicable) with an accounting of certain Accountable Disclosures (as defined below) of Protected Health Information made by the Covered Entity or its Business Associates. Under its Business Associate Agreements, Premier is contractually bound to provide a list of Accountable Disclosures made by Premier within certain time limits to permit the Covered Entity to meet its obligations under HIPAA. In the event that Premier makes an Accountable Disclosure or Covered Entity requests such an accounting, Premier will comply with the procedures set forth below and the applicable Business Associate Agreement.

2. Receipt of Request for an Accounting from a Covered Entity. Premier will direct Covered Entities to notify the appropriate Privacy Officer in writing if they desire to request an accounting of Disclosures of Protected Health Information that Premier has received on or on behalf of the Covered Entities. Premier Personnel who receive a request for an accounting of Disclosures of Protected Health Information will direct the request to the Premier Privacy Officer.

3. Receipt of a Request for an Accounting from an Individual. If any individual requests that Premier provide an accounting of Disclosures of Protected Health Information pertaining to such individual, Premier will forward the request to the Covered Entity from whom it received the Protected Health Information at issue. Premier will notify the Covered Entity in accordance with the timeframes in the Business Associate Agreement.

4. Types of Disclosures that are Accountable. "Accountable Disclosures" means all Disclosures of Protected Health Information made by Premier during the six (6) years prior to the date of the request (unless the Covered Entity requests a shorter time period) except for those Disclosures made

- i. to carry out Treatment, Payment, or Health Care Operations functions and administrative functions related to the patient's Payment;
- ii. to individuals about their own Protected Health Information;
- iii. incidental Uses or Disclosures as described in these Policies;
- iv. pursuant to an Authorization;
- v. for the Covered Entity's facility directory or to persons involved in the individual's care as described in these Policies;
- vi. for national security or intelligence purposes in accordance with the Privacy Standards;

vii. to correctional institutions or law enforcement officials in accordance with the Privacy Standards; or

viii. as part of a Limited Data Set

5. Content of the Accounting. Premier will provide, in writing, to the Covered Entity all of the following information with respect to each Accountable Disclosure:

i. the date of the Disclosure;

ii. the name of the entity or person who received the Protected Health Information and, if known, the address of such entity or person;

iii. a brief description of the Protected Health Information Disclosed;
and

iv. one of the following, as applicable:

(a) a brief statement of the purpose of the Disclosure that reasonably informs the Covered Entity of the basis for the Disclosure; or

(b) Entity's or Premier's compliance with HIPAA (if the Secretary's request relates to Accountable Disclosures).

6. Multiple Disclosures Exception. If, during the accounting period, Premier has made multiple Disclosures of Protected Health Information to the same person or entity for a single purpose, the Accounting may, with respect to such multiple Disclosures, contain the following:

i. the information listed above in the prior section with respect to the first Accountable Disclosure during the accounting period;

ii. the frequency, periodicity, or number of the Disclosures made during the accounting period; or

iii. the date of the last such Disclosure during the Accounting period.

7. Timing of Response to Request for Accounting. Premier will provide the Accounting of Accountable Disclosures within the time period required by the Business Associate Agreement, or, if no timeframe is specified, within a reasonable time period after receipt of a written request from the Covered Entity, which allows the Covered Entity to respond to the individual. If Premier does not maintain the Protected Health Information, but has knowledge that a Subcontractor of Premier maintains the information, Premier will request the Subcontractor to provide an accounting of any Accountable Disclosures relating to such Protected Health Information in accordance with these Policies. The Accounting should be made available by the Subcontractor to Premier within the timeframe set by the Downstream Subcontractor Agreement or a timeframe mutually agreeable to Premier and Covered Entity. If

Premier is not able to comply with the Covered Entity's request within the timeframe required by the Business Associate Agreement or an alternative deadline agreed upon by Premier and the Covered Entity, the Premier Privacy Officer will contact the Covered Entity to request an extension for action.

8. Request Log. The Premier Privacy Officer will maintain a request log and will keep information on each request for at least six (6) years from the date of the request. The request log will document the following with respect to each request for an accounting received:

- i. the date the request was received;
- ii. a copy of the request;
- iii. the date of Premier's response to the request;
- iv. a copy of the Accounting that was provided; and
- v. any other information that is pertinent to the request.

9. Responsibility of Responding to Individual's Request for Accounting. The provision of the Accounting of Accountable Disclosures to the Covered Entity's patient or beneficiary (as applicable), including the creation and/or maintenance of any notifications and/or documents in connection therewith, will be the sole responsibility of the Covered Entity.

F. Patient's Right to Request Restrictions on Certain Uses and Disclosures.

1. Individual Right. An individual has the right to request that a Covered Entity place additional restrictions on its Use and Disclosure of Protected Health Information for Treatment, Payment and Healthcare Operations. Except under limited circumstances set forth in these Policies, Covered Entities are not required to agree to a requested restriction from individuals.

2. Requests to Restrict Certain Uses and Disclosures. Covered Entities may agree to requests by patients or beneficiaries to restrict certain Uses and Disclosures of Protected Health Information. Premier accommodates a Covered Entity's reasonable request to assist it in complying with an agreed-upon individual's request to restrict Uses and Disclosures of Protected Health Information in accordance with this Policy, any commercial agreement and Business Associate Agreement with the Covered Entity, and Premier's corporate ethical and compliance obligations.

3. Conditions Pertaining to Special Communication Requirements. The Covered Entity's request must be provided to Premier in writing, and Premier will promptly respond to any such request.

4. Required Accommodations. A Covered Entity is required to comply with the patient's request for a restriction if:

i. Except as otherwise Required By Law, the Disclosure is to a Health Plan for purposes of carrying out Payment or Health Care Operations (and is not for the purposes of carrying out Treatment); and

ii. The Protected Health Information pertains solely to a health care item or service for which the Health Care Provider involved has been paid out of pocket in full.

G. Uses and Disclosures of Protected Health Information for Marketing.

1. General. Premier does not Use or Disclose an individual's Protected Health Information for Marketing a Covered Entity's or Premier's products or services unless in accordance with the Privacy Standards, Business Associate Agreement (if applicable), and these Policies. Under no circumstances will Premier Use or Disclose Highly Confidential Information for Marketing purposes unless permitted by Applicable Law governing such information or the Covered Entity has obtained a valid Authorization.

2. Procedures for Marketing Communications. If a Covered Entity directs Premier to make a communication on behalf of the Covered Entity that appears to have a Marketing purpose, the member of the Personnel responsible for the engagement with the Covered Entity shall forward the Covered Entity's request to the appropriate Privacy Officer, who will assess whether any proposed communication (whether oral, in writing or in any other medium) about a product or service of a Covered Entity, Premier or a third party encourages recipients of the communication to purchase or use the product or service and would involve the Use or Disclose Protected Health Information. If the communication encourages the purchase or use of the product or service and would involve the Use or Disclosure Protected Health Information, the appropriate Privacy Officer will then assess whether this Use or Disclosure constitutes Marketing, or whether one of the exceptions below applies.

3. Marketing Communications Pursuant to Authorization. If the Use or Disclosure constitutes Marketing, the appropriate Privacy Officer will ascertain whether the Covered Entity has obtained an Authorization from the patient or beneficiary, as applicable, that permits the Covered Entity (and its Business Associates) to Use or Disclose Protected Health Information for Marketing purposes. If the Covered Entity has a valid Authorization for Marketing, Premier will make the communication in accordance with the requirements of the Business Associate Agreement and the Authorization. If the Covered Entity does not have a valid Authorization from the patient or beneficiary (as applicable) to make the proposed communication, and the communication does satisfy one of the exceptions below, the communication may not be made until the Covered Entity secures a valid Authorization.

4. Exceptions to Definition of Marketing. The following communications to a third party that encourages the recipient of the communication to purchase or use the product or service and that would involve the Use or Disclose Protected Health Information are exempted from the requirements imposed on Marketing communications set forth in the Privacy Standards:

i. The communication describes a health-related product or service that is provided by Covered Entity *and* Covered Entity does *not* receive financial remuneration from a third party for the communication (provided that Covered Entity may receive payment for Treatment of an individual from a Health Plan or other responsible party).

ii. The communication is a refill reminder or other communication about a drug or biologic that is currently being prescribed for the individual (or a generic equivalent) and any financial remuneration received by Covered Entity or Premier in exchange for making the communication is reasonably related to the cost of making the communication.

iii. The communication is for Treatment of an individual by the Covered Entity (including case management or care coordination for the individual, or to direct or recommend alternative treatments, therapies, Health Care Providers, or settings of care to the individual) *and* the Covered Entity does *not* receive Financial Remuneration from a third party for the communication (provided that Covered Entity may receive payment for Treatment of an individual from a Health Plan or other responsible party).

iv. The communication is for case management or care coordination, contacting of individuals with information about treatment alternatives, and related functions to the extent these activities do not fall within the definition of Treatment *and* Covered Entity does *not* receive financial remuneration from a third party for the communication.

v. The communication is in the form of a face-to-face communication made by a Covered Entity to an individual.

vi. The communication is in the form of a promotional gift of nominal value of \$10 or less provided by the Covered Entity.

5. Applicable Law. Notwithstanding the exceptions set forth above, a Covered Entity may not Use or Disclose Protected Health Information for Marketing purposes unless also permitted by Applicable Law other than HIPAA.

6. Promotion of Premier's Products and Services. Premier will not Use or Disclose any Protected Health Information to make communications to promote Premier's products and services unless the communications are (1) consistent with the applicable Business Associate Agreement, (2) made on behalf of a Covered Entity in accordance with this Policy and (3) specifically approved by the appropriate Privacy Officer.

H. Uses and Disclosures of Protected Health Information for Fundraising.

1. Fundraising Communication. If the Covered Entity requests that Premier issue any fundraising communication to its patients or beneficiaries, Premier will confirm that the fundraising communication includes, in a clear and conspicuous manner, an opportunity for

the recipient of the communication to elect not to receive any further communication. Premier will not make fundraising communications to an individual who has elected not to receive such communications. Premier will not Use any Protected Health Information for its own fundraising purposes. Premier may only Use or Disclose the following information for fundraising purposes: (i) demographic information relating to an individual (including, without limitation, the patient's name, address, other contact information, age, gender, and date of birth); (ii) dates on which health care was provided to an individual by the Covered Entity; (iii) the department that treated the individual; (iv) the treating physician; (v) outcome information; and (vi) health insurance status.

2. Compliance with Applicable Business Associate Agreement. In all cases, if the Covered Entity directs Premier to make a fundraising communication on behalf of the Covered Entity, it must be consistent with the Business Associate Agreement between the parties.

I. Obligations Regarding Access and Amendment.

1. General. Covered Entities are required to provide individuals with access (e.g., copies) to Protected Health Information in medical records, billing records and other Designated Record Sets of the Covered Entities and to make appropriate amendments to the Designated Record Sets.

2. Maintenance of a Designated Record Set. Premier will not routinely maintain Designated Record Sets for its Covered Entity customers. In the event that Premier is requested to do so in writing, the responsible member of Personnel will first confer with the appropriate Privacy Officer. Premier will not proceed with this customer request unless approved by the Premier Privacy Officer.

3. Business Associate Agreement. In the event that Premier maintains a Designated Record Set of a Covered Entity, Premier will provide access to Protected Health Information in the time and manner specified in the Business Associate Agreement with the Covered Entity, consistent with these Policies and any corporate ethical and compliance standards.

J. Receipt of Request for Access or Amendment.

1. Request from Covered Entity. In the unlikely event that Premier maintains Protected Health Information in a Designated Record Set of a Covered Entity, Premier will respond to a Covered Entity request for access and amendment to certain Protected Health Information in accordance with the Business Associate Agreement with the Covered Entity and these Policies. Premier will direct Covered Entities to notify the appropriate Privacy Officer in writing if they desire to request access to or amendment of Protected Health Information that Premier has received on or on behalf of the Covered Entities. Premier employees who receive a request for Protected Health Information will direct the request to the appropriate Privacy Officer.

2. Request from Individual. If any individual requests that Premier provide access to or amend Protected Health Information pertaining to such individual, Premier will forward the request to the Covered Entity from whom it received the Protected Health Information at issue. The determination of whether to provide access to or amend Protected Health Information pursuant to a request from an individual and/or the denial of such request (including the creation and/or maintenance of any notification and/or creation of documents in connection therewith) will be the sole responsibility of the Covered Entity. In addition, the determination of whether all or part of the Protected Health Information is part of a Designated Record Set will be the sole responsibility of the Covered Entity.

3. Amendment Request Log. The appropriate Privacy Officer will maintain an access and amendment request log and will keep information on each request for at least six (6) years from the date of the request. The access and amendment request log will document the following information with respect to each request:

- i. the date the request was received;
 - ii. a copy of the written request (if any) or a general description of the request for information;
 - iii. the date of Premier's response to the request;
 - iv. a description of the content of Premier's response to the request;
- and
- v. any other information that is pertinent to the request.

K. Responding to Requests for Access to an Individual's Protected Health Information.

1. Timeframe for Providing Access to Covered Entity. Premier will respond to a written request from a Covered Entity for access to Protected Health Information in a Designated Record Set in accordance with any time frame set forth in the Business Associate Agreement with the Covered Entity or, if no time frame is specified, without unreasonable delay.

2. Timeframe for Providing Access to Covered Entity for Amendment. Premier will respond to a written request from a Covered Entity for amendment of Protected Health Information in a Designated Record Set in accordance with any time frame set forth in the Business Associate Agreement with the Covered Entity or, if no time frame is specified, without unreasonable delay.

3. Notifying Covered Entity of Subcontractor. If Premier does not maintain the Protected Health Information, but has knowledge that a Subcontractor of Premier maintains the information, Premier will notify the Covered Entity of the identity of such Subcontractor. Premier will ask the Subcontractor to comply with the Covered Entity's request unless the Covered Entity and Premier prefer that the Covered Entity contact the Subcontractor directly.

4. Notifying Covered Entity of Inability to Comply with Request. If Premier is not able to comply with the Covered Entity's request within the timeframe set forth in the Business Associate Agreement or an alternative timeframe agreed upon by Premier and the Covered Entity, the appropriate Privacy Officer will contact the Covered Entity to request an extension for action.

L. Responding to Requests for Amendment to Protected Health Information.

1. Incorporation of the Amendment. If the Covered Entity agrees to comply with a patient's or beneficiary's request for an amendment of Protected Health Information, Premier will incorporate the amendment designated by the Covered Entity in the Protected Health Information that Premier maintains. Premier will incorporate the amendment into the Protected Health Information in the form of a supplement to the Protected Health Information and will not delete any current information.

2. Amendment Denials. If the Covered Entity denies a patient's or beneficiary's request for access to or amendment of Protected Health Information, Premier will incorporate into the Protected Health Information any of the statements and/or documents that the Covered Entity provides to Premier relating to such denial.

3. Notification to Subcontractors of Premier of Approval/Denial of Amendments. If Protected Health Information maintained by a Subcontractor of Premier is the subject of a requested amendment, Premier will (1) notify the Subcontractor of the Covered Entity's approval and/or denial of the requested amendment, and (2) request the Subcontractor to accommodate any amendment and/or maintain documentation related to the denial of the amendment request in accordance with the applicable Downstream Subcontractor Agreement and this Policy to the extent practicable.

M. Interaction with Personal Representatives and Persons Involved in an Individual's Care

1. Covered Entity Instructions. Premier follows the Covered Entity's policies, procedures and instructions regarding interactions with a Covered Entity's patient's or beneficiary's Personal Representative and persons involved in the patient's care, including family members, other relatives, close personal friends of the patient or any other person identified by the patient or beneficiary, subject in each case to the approval of the appropriate Privacy Officer, which will be granted only after review and consultation with Premier legal counsel, as necessary.

2. Business Associate Agreement. Unless specifically set forth in the Business Associate Agreement, Premier will not directly interact with the Covered Entity's patients or beneficiaries. If the services that Premier provides for the Covered Entity require Premier to interact with patients or beneficiaries, Premier will follow the Covered Entity's policies and procedures with respect to a patient's or beneficiary's Personal Representative and persons involved in the individual's care, including family

members, other relatives, close personal friends or any other person identified by the patient or beneficiary.

3. Review by the Premier Privacy Officer. The appropriate Privacy Officer must review and approve the Covered Entity's policies and procedures before Personnel rely on them.

PH-4. Policies Regarding Use and Disclosure of Protected Health Information as a Covered Entity

A. Applicability. The Policies contained in this PH-4 (Policies Regarding Use and Disclosure of Protected Health Information as a Covered Entity) apply only where Premier is a Covered Entity.

B. Authorization for Use and Disclosure

1. General Procedure. Except as permitted by the Privacy Standards and these Policies, Premier may only use or disclose a patient's Protected Health Information if the patient or the patient's Personal Representative signs Premier's template authorization form or another authorization approved by the appropriate Privacy Officer. The appropriate Privacy Officer may approve an authorization form signed by the patient (which is not the template authorization form) if the authorization form contains each of the elements set forth below.

2. Elements of Patient Authorization. If a use or disclosure of Protected Health Information requires a patient's authorization, Premier may only make the use or disclosure pursuant to an authorization form written in plain language and containing the following elements:

i. Description of Health Information. The authorization must contain a description of the information to be used or disclosed that identifies the information in a specific and meaningful fashion. If Premier intends to use or disclose Highly Confidential Information, the patient must specifically authorize the use or disclosure of each type of Highly Confidential Information (*e.g.*, by checking the appropriate box on the template authorization form).

ii. Identification of Authorized Person. The authorization must contain the name or other specific identification of the person(s), or class of persons, authorized to make the requested use or disclosure.

iii. Identification of Recipient. The authorization must contain the name or other specific identification of the person(s), or class of persons, to whom Premier may make the requested use or disclosure.

iv. Description of Purpose(s). The authorization must contain a description of each purpose for which Protected Health Information is to be used or disclosed. This description must be specific enough to provide a patient with the facts that he/she needs to make an informed decision whether to allow release of the Protected Health Information. The statement "at the request of the individual" is a sufficient description of the purpose only when an individual initiates the authorization and does not (or elects not to) provide a statement of the purpose.

v. Expiration Date or Event. An authorization must contain an expiration date or an expiration event that relates to the patient or purpose of the use or

disclosure. If required by a law protecting Highly Confidential Information, the authorization must include an expiration date rather than an expiration event.

vi. Statement of Right to Revoke. The authorization must contain a statement of the patient's right to revoke the authorization in writing and either: (a) a statement of the exceptions to the patient's right to revoke an authorization and a description of how the patient may revoke the authorization; or (b) a reference to Premier's Notice of Privacy Practices, if the Notice of Privacy Practices describes the exceptions to the patient's right to revoke an authorization and the authorization revocation process.

vii. (In)Ability to Condition Treatment on the Authorization. The authorization must contain a statement that Premier may *not* condition its provision of health care to the patient on whether the patient signs the authorization, *unless* the health care to be provided is solely for the purpose of creating Protected Health Information to be disclosed to a third party and the patient's authorization permits Premier to release the patient's Protected Health Information to such third party.

viii. Re-disclosure. While state and federal law prohibits the re-disclosure of certain categories of records and information, the authorization must contain a statement that Protected Health Information used or disclosed pursuant to the authorization may be subject to re-disclosure by the recipient and may no longer be protected by the Privacy Standards.

ix. Dated Signature. The authorization must contain a signature of the patient or the patient's authorized Personal Representative and the date of the signature. If required by a law protecting Highly Confidential Information, the authorization must include the dated signature of a person who witnesses the patient's or Personal Representative's signature.

x. Personal Representative. If the authorization is signed by a Personal Representative of the patient, a description of such Personal Representative's authority to act for the patient must be included.

xi. Financial Remuneration for Marketing Activity. If the authorization is for a Marketing activity for which Premier receives any Financial Remuneration in connection with such Marketing activity, the authorization must state that Premier is receiving Financial Remuneration in connection with such Marketing activity.

xii. Financial or other Remuneration for Sale of Protected Health Information. If the authorization is for the Sale of Protected Health Information, the authorization must state that Premier is receiving remuneration from a third party for the disclosure of Protected Health Information to the third party.

3. Verification of Validity. Premier may not disclose Protected Health Information pursuant to an authorization unless the authorization meets the following requirements:

- i. Completion. An authorization form must contain all the elements identified above.
- ii. Not Expired. An authorization must not be expired.
- iii. Not Revoked. An authorization must not be revoked.
- iv. No Material False Information. An authorization must not contain any material information that Premier knows to be false.
- v. No Compound Authorizations. An authorization is not combined with any other document to create a compound authorization, except as set forth in the exceptions below:

(a) An authorization for the use or disclosure of Protected Health Information for a Research study may be combined with any other type of written permission for the same or another Research study. This exception includes combining an authorization for the use or disclosure of Protected Health Information for a Research study with an authorization for the creation or maintenance of a Research database or repository, or with a consent to participate in Research. Where Premier has conditioned the provision of Research-related Treatment on the provision of one of the authorizations, as permitted below, any compound authorization created under this Policy must clearly differentiate between the conditioned and unconditioned components and provide the patient with an opportunity to opt in to the Research activities described in the unconditioned authorization.

(b) An authorization for a use or disclosure of Psychotherapy Notes in accordance with PH-2.E may be combined with an authorization under this Section.

(c) An authorization covered under this Policy (other than an authorization for a use or disclosure of Psychotherapy Notes) may be combined with any other authorization covered under this Policy, except when Premier has conditioned the provision of health care on the provision of one of the authorizations. This prohibition on combining authorizations in circumstances where one authorization conditions the provision of health care on the provision of one of the authorizations does not apply to a combined authorization above.

(d) One authorization may be used to send the same information to two recipients. However, if different information is sent to two recipients, two separate authorizations should be obtained.

vi. No Conditioning Treatment on the Authorization. The provision of Treatment is not conditioned on whether the patient signs the authorization except as set forth below. Premier may condition the provision of Research-related Treatment on the provision of an authorization for the use or disclosure of Protected Health Information for such Research. Premier may condition the provision of Treatment if the sole purpose of the patient's Treatment is to provide Health Information to a third party (e.g., disclosure of the results of an employer-mandated drug test to the employer).

vii. Highly Confidential Information. If the authorization includes permission to disclose any Highly Confidential Information, the authorization must meet any additional requirements of the laws providing special protections for the Highly Confidential Information.

4. Revocation of Authorization

i. A patient may revoke an authorization at any time. To revoke an authorization the patient must submit the revocation in a writing that specifies the authorization to be revoked. A revocation will be effective immediately unless the patient specifies a future date in his or her written revocation. The revocation will not be valid where Premier has already acted in reliance upon the authorization.

5. Documentation Requirements

i. Authorization Stored in Patient Record. Premier must retain the original authorization and place it in the patient's record.

ii. Patient Will Receive Copy of Authorization. If Premier seeks an authorization from a patient for a use or disclosure of Protected Health Information, Premier must provide a copy of the patient's signed authorization form to the patient.

C. Uses and Disclosures to Carry Out Treatment, Payment and Health Care Operations by Premier as Covered Entity

i. General Procedure. Prior to using or disclosing Protected Health Information for Treatment, Payment or Health Care Operations purposes under this Policy, Premier must first determine whether the proposed use or disclosure is:

ii. an internal use by a member of Premier's Personnel for Premier's own Treatment, Payment or Health Care Operations;

iii. a disclosure to a third party (e.g., another Health Care Provider) for Premier's own Treatment activities;

iv. a disclosure to another Health Care Provider for the receiving Health Care Provider's Treatment activities;

- v. a disclosure to a third party (*e.g.*, a Health Plan or billing company) for Premier's own billing or other Payment purposes;
 - vi. a disclosure to another Covered Entity or non-Covered Entity Health Care Provider for the receiving Covered Entity's or Health Care Provider's Payment activities;
 - vii. a disclosure for Premier's own Health Care Operations purposes;
- or
- viii. a disclosure to another Health Care Provider for the receiving Health Care Provider's Health Care Operations.

After making this determination, Premier must follow the specific procedures applicable to the category of use or disclosure set forth below and any applicable laws governing Alcohol and Drug Abuse Treatment Program Records or other categories of Highly Confidential Information.

2. Use for Premier's Own Treatment, Payment and Health Care Operations. Premier's Personnel may use Protected Health Information internally in connection with Premier's own Treatment, Payment or Health Care Operations. (Uses for Payment and Health Care Operations purposes are subject to the minimum necessary standard set forth in PH-2.B.)

3. Disclosure for Treatment Purposes of Premier. Premier may disclose a patient's Protected Health Information as part of Premier's own Treatment activities.

4. Disclosure for Treatment Purposes of Another Health Care Provider. Premier may disclose a patient's Protected Health Information to another Health Care Provider for the receiving Health Care Provider's Treatment activities.

5. Disclosure for Premier's Payment Purposes. Premier may disclose a patient's Protected Health Information to an individual (*e.g.*, a minor-patient's parents) or a Health Plan in order to obtain payment for services provided to the patient. Before disclosing Protected Health Information to a billing agency, collection agency or other vendor assisting Premier with its Payment activities, Premier must enter into a contract with the vendor that includes the Business Associate contract provisions specified in these Policies.

6. Disclosure for Payment Purposes of Another Covered Entity or Health Care Provider. Premier may disclose Protected Health Information to another Covered Entity or a non-Covered Entity Health Care Provider for the billing or other Payment purposes of the Health Care Provider that receives the Protected Health Information.

7. Disclosure for Health Care Operations of Premier. Premier may disclose Protected Health Information to individual or entity (*i.e.*, a vendor) in order for the vendor to assist Premier with Premier's Health Care Operations. However, before disclosing Protected Health Information to a vendor assisting Premier with Premier's Health Care Operations,

Premier must enter into a Business Associate Agreement with the vendor in accordance with PH-1.B.

8. Disclosure for Health Care Operations Purposes of Another Covered Entity. Premier may disclose Protected Health Information to another Covered Entity for the Health Care Operations of the Covered Entity that receives the Protected Health Information, if all of the following conditions are met:

- i. Both Premier and the receiving Covered Entity either have or had a relationship with the patient who is the subject of the Protected Health Information being requested;
- ii. The Protected Health Information pertains to such relationship; and
- iii. The disclosure is either: (a) For the purpose of health care fraud and abuse detection or compliance; or (b) for any of the following purposes: (I) Conducting quality assessment and improvement activities, including outcomes evaluation and development of clinical guidelines, provided that research (*i.e.*, the obtaining of generalizable knowledge is not the primary purpose of any studies resulting from such activities); patient safety activities; population-based activities relating to improving health or reducing health care costs, protocol development, case management and care coordination, contacting of Health Care Providers and patients with information about Treatment alternatives; and related functions that do not include Treatment; or (II) Reviewing the competence or qualifications of health care professionals, evaluating practitioner and provider performance, health plan performance, conducting training programs in which students, trainees, or practitioners in areas of health care learn under supervision to practice or improve their skills as Health Care Providers, training of non-health care professionals, accreditation, certification, licensing, or credentialing activities.
- iv. The disclosure is consistent with any applicable laws governing Alcohol and Drug Abuse Treatment Program Records or other Highly Confidential Information.

D. Uses and Disclosures for Public Policy Purposes of Covered Entities

1. General Procedure. Under certain circumstances, the Privacy Standards permit Premier to use and disclose a patient's Protected Health Information without first obtaining the patient's authorization based on public policy considerations, as detailed in this policy. Before a Personnel member relies upon one of the public policy exceptions below, the appropriate Privacy Officer must first determine whether the proposed use or disclosure satisfies the requirements of the applicable exception. *If the proposed use or disclosure satisfies the exception, then Premier must also verify that any proposed use or disclosure of Highly Confidential Information is in accordance with the federal or state law regulating such information.*

2. Uses and Disclosures Required By Law. Premier may use or disclose Protected Health Information when required to do so by law, provided that the use or disclosure meets and is limited to the relevant requirements of such law. However, if such use or disclosure is covered under the categories of “Victims of Abuse, Neglect or Domestic Violence” (Section D.3.iii below), “Health Oversight” (Section D.3.iv below), “Judicial and Administrative Proceedings” (Section D.3.v below) or “Law Enforcement Officials” (Section D.3.vi below), then Premier must meet the requirements of Sections D.3.iii, D.3.iv, D.3.v or D.3.vi, as applicable.

3. Public Health Activities. Premier may use or disclose Protected Health Information for the following public health activities and purposes:

i. Public Health Reporting. To report Protected Health Information to public health authorities for the purpose of: (a) preventing or controlling disease, injury or disability; (b) the conduct of public health surveillance, public health investigations, and public health interventions; or (c) at the direction of a public health authority, to an official of a foreign government agency that is acting in collaboration with a public health authority.

ii. Child Abuse Reporting. To report child abuse and neglect to public health authorities or other government authorities authorized by law to receive such reports;

iii. FDA Reporting. To report information to a person subject to the jurisdiction of the U.S. Food and Drug Administration (“FDA”) about FDA-regulated products or activities for which that person has responsibility, for the purpose of activities related to the quality, safety or effectiveness of such FDA-regulated product or activity. Such purposes include: (a) to collect or report adverse events (or similar activities with respect to food or dietary supplements), product defects or problems (including problems with the use or labeling of a product), or biological product deviations; (b) to track FDA-regulated products; (c) to enable product recalls, repairs or replacement, or look-back (including locating and notifying individuals who have received products that have been recalled, withdrawn, or are the subject of look-back); or (d) conduct post-marketing surveillance;

iv. Notification of Exposure to Disease or Condition. To alert a person who may have been exposed to a communicable disease or may otherwise be at risk of contracting or spreading a disease or condition, if Premier is authorized by law to notify such person as necessary in the conduct of a public health intervention or investigation; or

v. Disclosures to Employers. To report information to the patient’s employer if:

(a) Premier provides a health care service to the patient at the request of the employer either to (a) conduct an evaluation relating to medical

surveillance of the workplace or (b) evaluate whether the patient has a work-related illness or injury;

(b) the Protected Health Information that is disclosed consists of findings concerning a work-related illness or injury or a workplace-related medical surveillance;

(c) the employer needs such findings in order to comply with its obligations under OSHA or the Mine Safety and Health Act, or under any state law having a similar purpose, to record such illness or injury or to carry out responsibilities for workplace medical surveillance; and

(d) Premier provides written notice to the patient that the Protected Health Information relating to the medical surveillance of the workplace and work-related illnesses and injuries is disclosed to the employer: (1) by giving a copy of the notice to the patient at the time the health care is provided; or (2) if the health care is provided on the worksite of the employer, by posting the notice in a prominent place at the location where the health care is provided.

vi. Disclosures to a School. To a school, about an individual who is a student or prospective student of the school, if:

(a) the Protected Health Information that is disclosed is limited to proof of immunization;

(b) the school is required by state or other law to have such proof of immunization prior to admitting the individual; and

(c) Premier obtains and documents the agreement to the disclosure from either: (1) a parent, guardian, or other person acting in loco parentis of the individual, if the individual is an unemancipated minor; or (2) the individual, if the individual is an adult or an emancipated minor.

4. Victims of Abuse, Neglect or Domestic Violence (Other than Child Abuse)

i. Except for child abuse reporting, covered under Section D.3.2 above, Premier may disclose Protected Health Information about a patient whom Premier reasonably believes to be a victim of abuse, neglect or domestic violence under one of the three circumstances described below:

(a) If the patient agrees to the disclosure;

(b) If Premier is Required By Law to make such disclosure; provided that the disclosure complies with and is limited to the relevant requirements of such law; or

(c) If Premier is expressly *permitted* by statute or regulation to make such disclosure and:

(a) Premier, in the exercise of professional judgment, believes the disclosure is necessary to prevent serious harm to the patient or other potential victims; or

(b) the patient is unable to agree to the disclosure because of incapacity, and a law enforcement or other public official authorized to receive the report represents that:

(1) the Protected Health Information for which disclosure is sought is not intended to be used against the patient; and

(2) an immediate enforcement activity that depends upon the disclosure would be materially and adversely affected by waiting until the patient is able to agree to the disclosure.

ii. Such disclosures may only be made to a government authority authorized by law to receive reports of abuse, neglect or domestic violence.

iii. Premier must inform the patient that a report has been (or will be) made, unless:

(a) Premier in the exercise of professional judgment, believes informing the patient would place the patient at risk of serious harm; or

(b) Premier would be informing a Personal Representative, and Premier reasonably believes the Personal Representative is responsible for the abuse, neglect or other injury, and that informing such person would not be in the best interests of the patient as determined by Premier, in the exercise of professional judgment.

5. Health Oversight Activities

i. Premier may disclose Protected Health Information to a health oversight agency for oversight activities authorized by law, including audits; civil, administrative or criminal investigations; inspections; licensure or disciplinary actions; civil, administrative or criminal proceedings or actions; or other activities necessary for appropriate oversight of:

(a) the health care system;

(b) government benefit programs for which Protected Health Information is relevant to beneficiary eligibility;

(c) entities subject to government regulatory programs for which Protected Health Information is necessary for determining compliance with program standards; or

(d) entities subject to civil rights laws for which Protected Health Information is necessary for determining compliance.

ii. For purposes of this exception, a health oversight activity does not include an investigation or other activity in which the patient is the subject of the investigation or activity and such investigation/activity is not related to the receipt of health care, a claim for public benefits related to health, or qualification for, or receipt of, public benefits or services when a patient's health is integral to the claim for public benefits or services.

6. Judicial and Administrative Proceedings

i. Premier staff must forward for handling any and all requests for Protected Health Information in connection with any judicial or administrative proceeding to the Covered Entity Privacy Officer immediately upon receipt. Premier may disclose Protected Health Information in the course of any judicial or administrative proceeding (so long as such disclosure is consistent with applicable federal and state law regulating Highly Confidential Information) as follows:

(a) in response to an order of a court or administrative tribunal, provided that Premier discloses only the information expressly authorized by such order; or

(b) in response to a subpoena, discovery request, or other lawful process, that is not accompanied by an order of a court or administrative tribunal, if one of the following two conditions is true:

(a) Premier receives satisfactory assurance from the party seeking the information that reasonable efforts have been made by such party to ensure that the patient who is the subject of the requested Protected Health Information has been given notice of the request. Premier has received "satisfactory assurance" if it has received from the party seeking the information a written statement and accompanying documentation demonstrating that:

(1) such party has made a good faith attempt to provide written notice to the patient (or if the patient's location is unknown, to mail a notice to the patient's last known address);

(2) the notice included sufficient information about the litigation or proceeding in which the Protected Health Information is requested to permit the patient to raise an objection to the court or administrative tribunal; and

(3) the time for the patient to raise objections to the court or administrative tribunal has elapsed and (i) no objections were filed, or (ii) all objections filed by the patient have been resolved by the court or the administrative tribunal and the disclosures being sought are consistent with such resolution.

(b) Premier receives satisfactory assurance from the party seeking the information that reasonable efforts have been made by such party to secure a qualified protective order (*i.e.*, an order of a court or of an administrative tribunal or a stipulation by the parties to the litigation or administrative proceeding that (a) prohibits the parties from using or disclosing the information for any purpose other than the litigation or proceeding for which such information was requested, and (b) requires the return to Premier or the destruction of the information and all copies thereof at the end of the litigation or proceeding). Premier has received “satisfactory assurance” if it has received from the party seeking the information a written statement and accompanying documentation demonstrating that:

(1) the parties to the dispute giving rise to the request for information have agreed to a qualified protective order and have presented it to the court or administrative tribunal with jurisdiction over the dispute; or

(2) the party seeking the Protected Health Information has requested a qualified protective order from such court or administrative tribunal.

ii. Premier also may disclose Protected Health Information in response to a subpoena, discovery request, or other lawful process without receiving satisfactory assurance from the requesting party, if Premier makes reasonable efforts to provide notice to the patient (in the manner prescribed above) or to seek a qualified protective order (in the manner prescribed by above).

7. Law Enforcement Officials. Subject to any federal or state law regulating Highly Confidential Information, Premier may disclose Protected Health Information for any of the six law enforcement purposes described below.

i. Premier may disclose Protected Health Information for a law enforcement purpose to a law enforcement official under the following circumstances:

(a) Required By Law. As Required By Law, including laws that require the reporting of certain types of wounds or other physical injuries;

(b) Response to Legal Process. In compliance with and as limited by the relevant requirements of:

- (a) a court order or court-ordered warrant, or a subpoena or summons issued by a judicial officer;
- (b) a grand jury subpoena;
- (c) an administrative request, including an administrative subpoena or summons, a civil or an authorized investigative demand, or similar process authorized under law, provided that:
 - (1) the information sought is relevant and material to a legitimate law enforcement inquiry;
 - (2) the request is specific and limited in scope to the extent reasonably practicable in light of the purpose for which the information is sought; and
 - (3) De-Identified Health Information could not reasonably be used.

ii. Identification or Location Purposes. In response to a law enforcement official's oral or written request for the purpose of identifying or locating a suspect, fugitive, material witness or missing person, Premier may disclose any of the following pieces of Protected Health Information:

- (a) name and address;
- (b) date and place of birth;
- (c) social security number;
- (d) ABO blood type and Rh factor;
- (e) type of injury;
- (f) date and time of treatment;
- (g) date and time of death, if applicable; and
- (h) a description of distinguishing physical characteristics, including height, weight, gender, race, hair and eye color, presence or absence of facial hair (*i.e.*, beard or mustache), scars and tattoos.

However, notwithstanding the previous sentence and list, Premier will not disclose any Protected Health Information (in response to a law enforcement official's oral or written request for the purpose of identifying or locating a suspect, fugitive, material witness or missing person) if the Protected Health

Information is related to the patient's DNA or DNA analysis, dental records, or typing, samples or analysis of body fluids or tissue.

iii. Information about Crime Victims. In response to a law enforcement official's request for information about a patient who is or is suspected to be a victim of a crime, Premier may disclose Protected Health Information provided:

(a) the disclosures are not authorized by, and subject to the conditions regarding public health activities under Section D.3 of this policy or victims of abuse, neglect or domestic violence under Section D.4 of this policy; and

(b) the patient agrees to the disclosure, or Premier is unable to obtain the patient's agreement because of incapacity or other emergency circumstance. If Premier is unable to obtain the patient's agreement because of incapacity or other emergency circumstance, then the following three conditions must be met:

(a) the law enforcement official represents that such information is needed to determine whether a violation of law by a person other than the victim has occurred, and such information is not intended to be used against the victim;

(b) the law enforcement official represents that immediate law enforcement activity that depends upon the disclosure would be materially and adversely affected by waiting until the patient is able to agree to the disclosure; and

(c) the disclosure is in the best interests of the patient as determined by a Premier pharmacist or other health care professional, in the exercise of professional judgment, taking into account the risk of further harm to the patient.

iv. Information about Decedents. Premier may disclose Protected Health Information about a patient who has died to a law enforcement official for the purpose of alerting law enforcement of the death of the patient if Premier has a suspicion that the death may have resulted from criminal conduct.

v. Information about Crime on Premises. Subject to the restrictions imposed by state and federal laws regulating the disclosure of Highly Confidential Information, Premier may disclose to a law enforcement official Protected Health Information that Premier believes in good faith constitutes evidence of criminal conduct that occurred on the premises of Premier.

vi. Information about Crime. Subject to the restrictions imposed by state and federal laws regulating the disclosure of Highly Confidential Information, Premier may disclose Protected Health Information to a law enforcement official if:

(a) Premier furnishes emergency health care in response to a medical emergency, other than on Premier's premises; and

(b) disclosure appears necessary to alert law enforcement to:

(a) the commission and nature of a crime;

(b) the location of such crime or of the victim(s) of such crime; and

(c) the identity, description and location of the perpetrator of such crime.

8. Decedents

i. Medical Examiner or Coroner. Premier may disclose Protected Health Information to a coroner or medical examiner for the purpose of identifying a deceased person, determining a cause of death, or other duties as authorized or required by applicable state law.

ii. Funeral Director. Premier may disclose Protected Health Information, except where restricted by state laws protecting Highly Confidential Information, to funeral directors, as necessary to carry out their duties with respect to decedents. If necessary for funeral directors to carry out their duties, Premier may disclose the Protected Health Information prior, and in reasonable anticipation of, the patient's death.

9. Organ and Tissue Procurement. Premier may disclose Protected Health Information to organ procurement organizations or other entities engaged in the procurement, banking, or transplantation of cadaveric organs, eyes, or tissue for the purpose of facilitating organ, eye or tissue donation and transplantation. This exception does not allow disclosures in connection with live donor activities. In live donor situations, an authorization will be required.

10. To Avert a Serious Threat to Health or Safety

i. Premier may use or disclose Protected Health Information, but not information about HIV-related testing and substance abuse treatment program records and information:

(a) if Premier, in good faith, believes the use or disclosure is necessary to prevent or lessen a serious and imminent threat to the health or safety of a person or the public and the use or disclosure is to a person or persons reasonably able to prevent or lessen the threat, including the target of the threat; or

(b) the use or disclosure is necessary for law enforcement authorities to identify or apprehend a patient:

(a) where it appears from all the circumstances that the patient has escaped from a correctional institution or from lawful custody; or

(b) because of a statement by a patient admitting participation in a violent crime that Premier reasonably believes may have caused serious physical harm to the victim, provided:

(1) no use or disclosure is permitted pursuant to this exception if Premier became aware of the statement either:

a) in the course of treatment to affect the propensity to commit the criminal conduct that is the basis for the disclosure, or counseling or therapy, or

b) through a request by the patient to initiate or to be referred for the treatment, counseling or therapy.

(2) a disclosure made pursuant to this exception will contain only the statement admitting participation in a violent crime that Premier reasonably believes may have caused serious physical harm to the victim and the eight items of Protected Health Information otherwise allowed for identification purposes in response to law enforcement requests.

ii. Presumption of good faith belief. If Premier uses or discloses Protected Health Information pursuant to this exception, it will be presumed to have acted in good faith with regard to a belief that the criteria for disclosure were satisfied, if the belief is based upon Premier's actual knowledge or in reliance on a credible representation by a person with apparent knowledge or authority.

11. Specialized Government Functions.

i. Military and Veterans Activities.

(a) Premier may use and disclose the Protected Health Information of the patients who are Armed Forces personnel for activities deemed necessary by appropriate military command authorities to assure the proper execution of the military mission, if the appropriate military authority has published by notice in the Federal Register the following information:

(a) appropriate military command authorities; and

(b) the purposes for which the Protected Health Information may be used or disclosed.

(b) Premier may use and disclose the Protected Health Information of patients who are foreign military personnel to their appropriate foreign military authority for the same purposes for which uses and disclosure are permitted for Armed Forces personnel.

(c) Other uses and disclosures regarding such patients are subject to the general HIPAA privacy requirements addressed in these policies.

ii. National Security and Intelligence Activities. Premier may disclose Protected Health Information to authorized federal officials for the conduct of lawful intelligence, counter-intelligence, and other national security activities authorized by the National Security Act and implementing authority (for example, Executive Order 12333).

iii. Protective Services for the President and Others. Premier may disclose Protected Health Information to authorized federal officials for the provisions of protective services to the President or other persons as authorized by 18 U.S.C. § 3056, or to foreign heads of state or other persons authorized by 22 U.S.C. § 2709(a)(3), or for the conduct of investigations authorized by 18 U.S.C. §§ 871 and 879.

12. Correctional Institutions. Premier may disclose Protected Health Information about an inmate or other individual to a correctional institution or a law enforcement official having lawful custody of an inmate or other individual if the correctional institution or such law enforcement official represents that such health information is necessary for:

- i. the provision of health care to such individuals;
- ii. the health and safety of such individual or other inmates;
- iii. the health and safety of such individuals and officers or other persons responsible for the transporting of inmates or their transfer from one institution, facility or setting to another;
- iv. law enforcement on the premises of the correctional institution; or
- v. the administration and maintenance of the safety, security and good order of the correctional institution.

13. Workers' Compensation. Premier may disclose Protected Health Information as authorized by, and to the extent necessary to comply with, laws relating to workers' compensation or other similar programs.

E. Uses and Disclosures to Persons Involved in the Patient's Care and for Notification Purposes.

1. General Policy. Premier may disclose portions of a patient's Protected Health Information to certain persons involved in the patient's care or Payment for such care. In addition, Premier may use or disclose the patient's Protected Health Information in order to

notify (or assist in notifying) certain persons of the patient's location, general condition or death. In both instances, except in limited circumstances described in Section E.5 below, Premier must provide the patient an opportunity to object or agree to the use or disclosure prior to using or disclosing such Protected Health Information.

2. General Rule. Absent an objection by the patient (and without having to obtain the patient's Authorization), Premier may:

- i. Disclose to a Family Member, other relative, or close personal friend of the patient, or any other person identified by the patient, Protected Health Information which is directly relevant to such person's involvement with the patient's health care or Payment related to the patient's care; or
- ii. Use or disclose the patient's Protected Health Information to notify (or assist in the notification of) the patient's Family Member, Personal Representative or another person responsible for the patient's care of the patient's location, general condition or death. In connection with this purpose, Premier may disclose the patient's Protected Health Information to public or private entities authorized by law or its charter to assist in disaster relief efforts in order to coordinate the notification efforts described in this Section D.2.

3. Limitations on the Protected Health Information Used or Disclosed

- i. Premier may disclose only Protected Health Information that is relevant to a Family Member's, friend's or other person's involvement in the patient's health care or Payment for such care. Further, Premier may not disclose Protected Health Information that is not relevant to the patient's current condition and that could prove embarrassing to the patient.
- ii. Unless the patient consents in writing to such disclosure, Premier will not disclose to anyone information relating to Alcohol and Drug Abuse Treatment Program services provided to the patient by Premier or another program.
- iii. Premier staff should not assume that a patient's agreement or lack of objection pursuant to Section E.4 below implies agreement to disclose Protected Health Information indefinitely in the future.

4. Uses and Disclosures Permitted Where Patient Agrees to Disclosure.

Where a patient is present for, or otherwise available prior to a disclosure made pursuant to Section E.2 above, and is capable of agreeing to the disclosure, Premier staff may only disclose the patient's Protected Health Information (subject to the limitations described in Section E.3 above), in any of the following situations:

- i. Agreement. The patient agrees to the disclosure; or
- ii. No Objection. The patient does not express an objection to the disclosure when given the opportunity to do so; or

iii. Reasonable Inference. A pharmacist or other health care professional reasonably infers from the circumstances, based on the exercise of professional judgment, that the patient does not object to the disclosure.

(a) When possible, staff should formally ask the patient (outside the presence of the Family Member, close personal friend, etc.) whether the patient objects to such person's presence during a procedure or discussion.

(b) Reliance on inferences set forth in this Section should be as infrequent as practicable.

(c) *Example of reasonable inference:* if a patient's Family Member is in the physical presence of the patient as the patient discusses his or her medication with a pharmacist, the pharmacist can infer that disclosures to the Family Member are appropriate.

5. Uses and Disclosures Permitted When Patient Is Unable To Agree But Disclosure is in Patient's Best Interest. Where a patient is not available or cannot agree or object to a use or disclosure to be made pursuant to Section E.2 above because of incapacity or emergency, staff may disclose Protected Health Information (subject to the limitations described in Section E.3 above) as follows:

i. Staff determines, in the exercise of professional judgment, that the disclosure is in the best interest of the patient. (For example, pharmacy staff may infer that it is in the best interests of patient to allow another person to pick up a prescription on behalf of patient.)

ii. The disclosure is limited only to Protected Health Information directly relevant to the person's involvement in the patient's health care or payment related to the person's health care or needed for notification purposes.

iii. *Disclosures over the telephone.* In general, Personnel should not give patient status information to a person over the telephone. Personnel may release Protected Health Information over the telephone only in very limited circumstances. The first circumstance is when the Personnel member recognizes the voice of a person who had previously been identified by a patient. The second is in order to facilitate immediate Treatment or to interpret the health care practitioner's instructions to a person who is assisting the patient, only after the health care practitioner has determined that the patient is unavailable (and therefore cannot give consent) and has determined that it is in the best interest of the patient to disclose the Protected Health Information. The Protected Health Information disclosed over the telephone should be limited to the information directly relevant to the person's involvement in the patient's care. If more detailed information is requested, the health care practitioner should request a call back by the patient and the person requesting the information. Premier may implement reasonable alternative measures for disclosing Protected Health Information over the telephone, including, for

example, the use of a security code or password, identifying number, or mother's maiden name.

6. Uses and Disclosures Permitted When the Person is Deceased. If the individual is deceased, Premier may disclose to a Family Member, other relative, a close personal friend of the individual, a Personal Representative of the individual, another person who was responsible for the care of the individual or any other person that was identified by the individual who were involved in the individual's care or payment for health care prior to the individual's death, Protected Health Information of the individual that is relevant to such persons involvement, unless doing so is inconsistent with any prior expressed preference of the individual that is known to Premier.

7. Verification of Identity. Premier is not required to verify formally the identity of the Family Members or other individuals involved in the patient's care under the procedures delineated in PH-2.H. The patient's actions involving a person in his or her care serves to verify that person's identity for purposes of this policy.

8. Documentation. Premier may (a) orally inform the patient of his or her opportunity to object to uses and disclosures of Protected Health Information to a person involved in the patient's care and (b) obtain the patient's oral agreement or objection. However, Premier may obtain written permission to make the uses and disclosures described in this policy.

9. Questions About Interpretation. If any member of the Personnel is concerned about the immediate interpretation of this policy, that staff member is directed to not release information upon an inquiry from a patient's family or friends, and to refer the issue to the appropriate Privacy Officer for resolution.

F. Uses and Disclosures of Protected Health Information for Marketing

1. General Procedure. Premier does not use or disclose a patient's Protected Health Information for Marketing Premier's or a third party's products or services unless in accordance with this Policy. Premier does not use or disclose Highly Confidential Information for Marketing unless permitted by the law placing special privacy protections on the information or Premier has obtained a valid Authorization.

2. Assessment of Proposed Communication. The appropriate Privacy Officer will first assess whether any proposed communication (whether oral, in writing or in any other medium) about a product or service of Premier or a third party encourages recipients of the communication to purchase or use the product or service. Before Premier makes the proposed communication, the appropriate Privacy Officer will confirm that either (1) the communication meets an exception to the marketing authorization requirement in Section F.3 below or (2) Premier has obtained a valid authorization consistent with Policy # P-10 (Patient Authorization for Use and Disclosure of Protected Health Information).

3. Permissible Communications Without a Valid Authorization. Premier may make a communication about a product or service that encourages the recipient of the

communication to purchase or use the product or service (without the patient's authorization) if the appropriate Privacy Officer determines that the proposed communication meets the conditions of any of the following exceptions (and, if the communication involves any Highly Confidential Information any laws applicable to such information):

i. *Refill Reminder.* The communication is a refill reminder or other communication about a drug or biologic that is currently being prescribed for the individual (or a generic equivalent) and any Financial Remuneration received by Premier in exchange for making the communication is reasonably related to Premier's cost of making the communication.

ii. *Treatment and No Financial Remuneration.* The communication is for Treatment of an individual by Premier (including case management or care coordination for the individual, or to direct or recommend alternative treatments, therapies, Health Care Providers, or settings of care to the individual) *and* Premier does *not* receive Financial Remuneration from a third party for the communication (provided that Premier may receive payment for Treatment of an individual from a Health Plan or other responsible party).

iii. *Health-Related Product and No Financial Remuneration.* The communication describes a health-related product or service that is provided by Premier *and* Premier does *not* receive Financial Remuneration from a third party for the communication (provided that Premier may receive payment for Treatment of an individual from a Health Plan or other responsible party); or

iv. *Case Management or Care Coordination and No Financial Remuneration.* The communication is for case management or care coordination, contacting of individuals with information about treatment alternatives, and related functions to the extent these activities do not fall within the definition of Treatment *and* Premier does *not* receive Financial Remuneration from a third party for the communication.

v. *Face-to-Face Communication.* Premier may make a Marketing communication without an Authorization if the Marketing communication is in the form of a face-to-face communication to an individual.

vi. *Gifts of Nominal Value.* Premier may make a Marketing communication in the form of a promotional gift of nominal value of \$10 or less, provided that the Compliance Officer approves the gift.

G. Uses and Disclosures of Protected Health Information for Fundraising

1. General Policy. Premier obtains a patient's authorization before using or disclosing the patient's Protected Health Information for fundraising purposes unless an exception exists under this Policy. Further, under no circumstances may Premier use or disclose Highly Confidential Information for fundraising purposes unless permitted by applicable state or

federal law regulating the applicable category of Highly Confidential Information or the patient's signed Authorization.

2. General Procedures. Premier may use or disclose Protected Health Information about a patient for fundraising purposes *only* if it has first obtained the patient's valid Authorization or as permitted in Section G.3 below.

3. Permitted Uses and Disclosures of Protected Health Information for Fundraising. Premier may use, or disclose to a Business Associate or to a not-for-profit charitable foundation (which entity is affiliated with Premier and has been formed, at least in part, for the purpose of supporting Premier), the following Protected Health Information to raise funds for Premier's own benefit without first obtaining a patient's authorization:

- i. demographic information relating to a patient, including, without limitation, the patient's name, address, other contact information, age, gender, and date of birth;
- ii. dates of health care services provided to the patient;
- iii. the department that treated the patient;
- iv. treating physician;
- v. outcome information; and
- vi. health insurance status.

4. Use and Disclosure of Highly Confidential Information for Fundraising. Premier may not use or disclose Highly Confidential Information for fundraising purpose unless permitted by applicable state or other federal law regulating the applicable category of Highly Confidential Information or the patient's signed Authorization.

5. Notification to Patient. Premier will include in its "Notice of Privacy Practices," a statement that Premier may use the information included in Section G.3.i-vi above for fundraising purposes without first obtaining the patient's authorization and that the patient has a right to opt out of receiving fundraising communications from Premier.

6. Opt Out Provision. With each fundraising communication made to a patient, Premier will provide the individual with a clear and conspicuous opportunity to elect not to receive any further fundraising communications. The "opt out" may be limited only to further communications for a particular fundraising campaign or Premier may provide the patient with an opportunity to opt out of all future fundraising communications. Premier will not make fundraising communications to a patient who has elected not to receive such communications. The method for a patient to elect not to receive further fundraising communications may not cause the patient to incur an undue burden or more than a nominal cost. An email address, phone number, business reply card or similar form of communication is an acceptable method for a patient to elect not to receive further fundraising communications. Premier will not require a

patient to write and send a letter to Premier to opt out of fundraising communications. Premier will not condition treatment or payment with respect to a patient on the patient's choice regarding the receipt of fundraising communications.

7. Opportunity to Opt Back In. Premier may provide a patient who has elected not to receive further fundraising communications with a method to opt back in to receive such communications should the patient wish to do so.

H. Uses and Disclosures of Protected Health Information for Research

1. General Policy. Premier may only use or disclosure an individual's Protected Health Information for Research purposes in accordance with this Policy. In addition, Premier must also conduct Research in accordance with other federal and state laws governing human subjects research (collectively, "Applicable Research Laws") and Premier's policies to assure compliance with Applicable Research Laws.

2. Research Pathways. Premier does not use or disclose an individual's Protected Health Information for Research purposes unless the use or disclosure satisfies the requirements of one of the following Research pathways (and otherwise in compliance with Applicable Research Laws):

i. The use or disclosure is Preparatory to Research in accordance with Section H.3 below;

ii. The Protected Health Information is about decedents in accordance with Section H.4 below;

iii. The Protected Health Information is used or disclosed in a Limited Data Set in accordance with Section H.5 below;

iv. The Protected Health Information has been de-identified in accordance with Section H.6 below;

v. An IRB or a Privacy Board has approved a waiver of Authorization in accordance with Section H.7 below; or

vi. Premier has an authorization for the use or disclosure from the individual who is the subject of the Protected Health Information and the Authorization complies with Section H.8.

3. Preparatory for Research. Protected Health Information may be used by or disclosed to a researcher to engage in activities preparatory to research, such as the development of a research protocol, identification (but not contacting) potentially eligible individuals, or for similar purposes preparatory to Research provided the researcher represents to Premier that: (1) the use or disclosure is sought solely for such purposes, (2) no Protected Health Information will be removed from Premier's physical or electronic premises by the researcher in the course

of the review, and (3) the Protected Health Information for which use or access is sought is necessary for the research purposes.

4. Research on Decedents. A researcher may use or disclose Protected Health Information for Research on decedents provided the researcher: (i) represents to Premier that the use or disclosure is sought solely for Research on the Protected Health Information of decedents, (ii) provides to Premier, upon request, documentation of the death of the individual, and (iii) represents to Premier that the Protected Health Information is necessary for the Research.

5. Limited Data Set. Premier may use and disclose Protected Health Information for Research if an IRB or Privacy Board determines that the health information is a Limited Data Set in accordance with PH-2.I.

6. De-Identification. Premier may use and disclose health information for Research if an IRB or privacy board determines that the health information is de-identified in accordance with PH-2.I.

7. Waiver of Authorization: An IRB or Privacy Board may approve a waiver or alteration of the Privacy Standard's authorization requirement if it documents the following:

i. The identity of the IRB or Privacy Board approving the waiver or alteration.

ii. The date of the approval of the waiver or alteration.

iii. That the following criteria for the waiver or alteration have been met:

(a) The use or disclosure of Protected Health Information involves no more than minimal risk to the individuals or their privacy, based on

(a) an adequate plan to protect identifiers from improper use and disclosure,

(b) an adequate plan to destroy the identifiers at the earliest opportunity (unless there is a health or research justification for retaining identifiers or such retention is otherwise Required By Law) and

(c) adequate assurances that the Protected Health Information will not be reused or disclosed to any other person or entity except as Required By Law, for authorized oversight of the Research project, or for other Research permitted under this policy.

(b) The Research could not practicably be conducted without the alteration or waiver;

(c) The Research could not practicably be conducted without access to and use of the Protected Health Information;

(d) A brief description of the Protected Health Information to be used or disclosed;

(e) A statement that the alteration or waiver of authorization has been reviewed and approved by the IRB or Privacy Board in accordance with normal or expedited procedures under the Privacy Standards.

iv. The signature of the Chair or other member, as designated by the Chair, of the IRB or Privacy Board.

8. Authorization. Unless another Research pathway is available, Premier must obtain an individual's Authorization to use or disclosure Protected Health Information about the individual for Research purposes in accordance with this Section.

i. Valid Authorization for Future Use. In describing the purposes for which the Protected Health Information may be used or disclosed in connection with Research, an authorization may describe future Research and does not have to be study specific, provided however, that the authorization satisfies all of the following requirements:

(a) The supervising IRB determines that authorization for future use, as opposed to a study-specific authorization, is appropriate.

(b) The Authorization satisfies all of the requirements under PH-4.B with respect to the future Research, even if described in a more general manner.

(c) If the future use may include the use and/or disclosure of Protected Health Information collected beyond the time of the main study (not the future use Research-related activity), this Protected Health Information is specifically discussed.

(d) The purpose must adequately describe the future purposes such that it would be reasonable for the individual to expect that his/her Protected Health Information could be used or disclosed for this future Research.

(e) As may be required by Applicable Research Laws or the determination of the supervising IRB, the authorization describes any Highly Confidential Information that will be used or disclosed in the future research;

At its discretion, Premier's IRB may develop guidelines for evaluating authorizations that involve future Research-related activities.

ii. Compound Research Authorizations. A valid authorization may combine conditional research components and unconditional research components into a single authorization, provided that the authorization satisfies the requirements of this Section. For the purposes of this Policy, a “conditional research component” is a use or disclosure of Protected Health Information in connection with a Research study to which a individual must agree as a condition of participating in the study (for example, use and disclosure of Protected Health Information relating to adverse events experienced by the individual). For the purposes of this policy, an “unconditional research component” is a use or disclosure of Protected Health Information in connection with a Research study to which an individual need not agree as a condition of participating in the study (for example, use of Protected Health Information to look for a related biomarker). A research Authorization that combines conditional and unconditional components is “Compound Research Authorization.” Premier may employ a Compound Research Authorization.

(a) The supervising IRB determines that a Compound Research Authorization is appropriate, as opposed to separate authorizations for the conditional and unconditional components;

(b) The Compound Research Authorization may only combine authorization to use or disclose psychotherapy notes in connection with research with an authorization for another use or disclosure of psychotherapy notes;

(c) The Compound Research Authorization employs an “opt-in” model in which individuals have to affirmatively indicate their interest in participating in the unconditional research components; *and*

(d) In the event that the IRB determines that the unconditional components should be detailed in a separate form or brochure, that separate information sheet or brochure is referenced in the Compound Research Authorization and is considered part of the Compound Research Authorization.

At its discretion, Premier’s IRB may develop guidelines for evaluating authorizations that involve future Research-related activities.

iii. Revocation. The revocation provisions in an authorization that incorporates future use or in a Compound Research Authorization must meet the standard revocation requirements of a valid authorization. In addition, when it is clear that an individual is revoking only a part of an authorization, for example, future use or an unconditional research component, the partial revocation does not result in a revocation of the complete authorization. If it is not clear, however, Premier must obtain written clarification from the subject as to whether the revocation is partial or complete. If such clarification is not obtained, the revocation will be deemed a complete revocation.

I. Notice of Privacy Practices.

1. General Rule. Premier recognizes a patient's right to receive adequate notice of the uses and disclosures of Protected Health Information that may be made by Premier and of the patient's rights and Premier's legal duties with respect to Protected Health Information.

2. Content of Notice. Premier provides each patient with which it Premier has a direct treatment relationship, including without limitation Commcare patients, with a notice of Premier's or Commcare's privacy practices that is written in plain language and that contains the elements required by the Privacy Standards. The appropriate Privacy Officer maintains the current version of Premier's and Commcare's Notice of Privacy Practices (the "Notice") in the Privacy Office.

3. Acknowledgment of Receipt. Premier uses good faith effort to obtain a written or electronic acknowledgment by the patient of receipt or electronic viewing of the Notice. The written acknowledgement may take any reasonable form such as patient initialing the notice, signing a list, executing a separate document, or clicking an acknowledgement on a website. In any event, the language states the following or similar: "By signing below, I hereby acknowledge receipt of Premier's Notice of Privacy Practices". If Premier obtains written or electronic acknowledgment by the patient of receipt or electronic viewing of the Notice, Premier maintains documentation of the acknowledgment for the documentation period specified below. If despite good faith efforts, Premier is unable to obtain a written acknowledgment of receipt, then Premier will document its efforts and the reason(s) why the written acknowledgment of receipt could not be obtained.

4. Revisions to the Notice. Premier will promptly revise and distribute its Notice whenever there is a material change to the uses or disclosures, the individual's rights, Premier's legal duties, or other privacy practices stated in the Notice. If the Notice permits Premier to make changes to its terms at any time, a material change to the Notice may be implemented at any time. If the Notice does not permit such changes at any time, a material change to the Notice will not be implemented prior to the effective date of the Notice in which such material change is reflected.

5. Provision of Notice.

- i. Premier makes the Notice available upon request to any person.
- ii. Premier provides the Notice electronically or in person no later than the date of the first service delivery, including service delivered electronically.
- iii. Premier makes the Notice available at its offices for individuals to take with them upon request.
- iv. Premier posts the Notice in a clear and prominent location where it is reasonable to expect individuals seeking service from Premier to be able to read the Notice.

v. Premier's Covered Entity provides the Notice to each of its patients through a general posting on the Covered Entity's website and by inclusion with prescription medications that are shipped to the patient. Premier's Covered Entity provides revised Notices to its patients through a general posting on the Covered Entity's website and by inclusion in the next prescription medications that are shipped to the patient.

6. Electronic Notice.

i. Premier prominently posts its Notice on its website and makes the Notice available electronically through the website. Premier should coordinate the Notice language with Premier's website privacy policy and terms of use.

ii. Premier may provide the Notice to a patient by e-mail, if the patient agrees to electronic notice and such agreement has not been withdrawn. Premier retains confirmation of the transmission of the e-mail. If Premier knows that the e-mail transmission has failed, the Notice will be provided to the patient by other means.

iii. If the first service delivery to a patient is delivered electronically, Premier must provide electronic notice automatically and contemporaneously in response to the patient's first request for service.

iv. The patient who is the recipient of electronic notice may obtain a paper copy of the Notice from Premier upon request.

7. Documentation. Premier documents compliance with the Notice requirements by retaining copies of the Notices it issues and any written acknowledgments of receipt of the Notice (or documentation of good faith efforts to obtain such acknowledgment) for a period of six years from the date of their creation, or the date when such Notice last was in effect, whichever is later.

J. Designated Record Sets.

1. Designated Record Sets Defined. As used in these policies, the term "Designated Record Set" means a group of records maintained by or for Premier that is (i) the medical records and billing records about individuals maintained by or for Premier, (ii) the enrollment, Payment, claims adjudication, and case or medical management record systems maintained by or for a Health Plan; or (iii) used, in whole or in part, by or for Premier to make decisions about individuals. As used herein, the term "Record" means any item, collection, or grouping of information that includes Protected Health Information and is maintained, collected, used, or disseminated by or for the Covered Entity.

2. Premier Designated Record Sets. Premier maintains the following Designated Record Sets containing Protected Health Information about patients:

i. Medical records;

- ii. Billing records;
- iii. other records used, in whole or in part, by or for Premier to make decisions about patients.

K. Patient's Right to Access Protected Health Information.

1. General Policy. It is the policy of Premier to recognize the right of a patient to have access to (*i.e.*, inspect and obtain a copy of) Protected Health Information about the patient which is maintained by Premier in a Designated Record Set for as long as the Protected Health Information is maintained in the Designated Record Set. The patient will have a right of access to any Protected Health Information that is used, in whole or in part, to make decisions about him or her, *except* for Protected Health Information listed in Section K.4 below and in situations in which Premier has determined that access would be inappropriate in accordance with Section K.5 below.

2. Request for Access Forms. Premier will make available to patients on request an Access Request Form notifying patient that, except in certain circumstances below, he/she has the right of access to any Protected Health Information maintained in a Designated Record Set. Additionally, this Access Request Form will explain Premier's procedures for obtaining such Protected Health Information.

3. Request for Access in Writing. A request by a patient to inspect or to obtain a copy of the Protected Health Information (a "Request") may be in writing and set forth on Premier's Access Request Form that has been approved by the appropriate Privacy Officer or submitted orally and transcribed onto the Access Request Form.

4. Patient Right of Access. The patient will have a right of access (*i.e.*, a right to inspect and obtain a copy of) to Protected Health Information about the patient which is maintained by Premier in a Designated Record Set for as long as the Protected Health Information is maintained in the Designated Record Set except for information compiled in reasonable anticipation of, or for use in, a civil, criminal, or administrative action or proceeding.

5. Action on a Request.

i. Timing of Action on Request. Premier will act on the Request within 30 days of receiving the Request (unless state law requires a quicker response time). If Premier is unable to act on a Request for on-site records within 30 days and permitted to do so under state law, it may extend the deadline once by no more than 30 days by providing the patient with a written statement indicating the reasons for the delay and the date by which Premier will complete its action on the Request.

ii. Types of Actions on a Request. Premier may take one of the following four actions on a request:

- (a) determine that the patient has no right to access as set forth in this policy;

- (b) provide access in accordance with this policy;
- (c) deny the Request without an opportunity for review as set below; or
- (d) deny the Request and give the patient an opportunity to obtain review of the denial as set forth below.

iii. Access Procedures. If Premier provides access to the patient (in whole or in part):

(a) Timing and Manner of Access. Premier must notify the patient of the decision and arrange for a mutually convenient time and place to provide the access requested. Once Premier has located the requested Protected Health Information, the individual has the right to inspect the information, copy the information or to do both. If a patient's request for access directs Premier to transmit the copy of Protected Health Information directly to another person designated by the patient, Premier will only provide the copy to the designee if the request: (i) is in writing; (ii) is signed by the patient; (iii) clearly identifies the designated person; and (iv) clearly identifies where to send the copy of Protected Health Information.

(b) Format. Premier will provide the requested Protected Health Information in the form and format requested, provided that the information is readily producible in such form or format. If the Protected Health Information is not readily producible in the requested format, Premier will produce a hard copy of the information or in another form and format to which the patient and Premier can agree. If the Protected Health Information that is the subject of a request for access is maintained in one or more Designated Record Sets electronically and if the patient requests an electronic copy of such information, Premier must provide the patient with access to the Protected Health Information in the electronic form and format requested by the individual, if it is readily producible in such form and format; or, if not, in a readable electronic form and format as agreed to by Premier and the patient.

(c) Mailing Copies. If the patient requests that Premier mail a paper copy of the requested Protected Health Information, Premier will do so. If the patient requests to inspect Protected Health Information that is maintained electronically, Premier will print out a copy and allow the patient to view the print-out on-site.

(d) Access to Electronic Health Record. If the individual requests Protected Health Information that Premier uses or maintains in an Electronic Health Record, the individual has the right to obtain a copy of such information in an electronic format and, if the individual chooses, to direct Premier to transmit such copy directly to an entity or person designated by the

individual, provided that any such choice is clear, conspicuous, and specific. Any fee that Premier may impose for providing such individual with a copy of such information (or a summary or explanation of such information) if such copy (or summary or explanation) is in an electronic form shall not be greater than Premier's labor costs in responding to the request for the copy (or summary or explanation).

(e) Duplicate Records. If the same Protected Health Information is maintained in more than one Designated Record Set or at more than one location, Premier need only produce the information once per Request.

(f) Summary. Premier may provide the patient with a summary of the Protected Health Information requested, in lieu of providing an opportunity to inspect or obtain a copy of the Protected Health Information, if the patient agrees in advance (i) to the provision of the summary, (ii) to any fees imposed by Premier for such summary and (iii) to any extended time period required by Premier to produce the summary.

iv. Permissible Grounds for Denial of Access Without Opportunity for Review. Premier may (but is not required to) deny a Request without providing the patient with a right to have the denial reviewed for any of the following three reasons:

(a) If the requested Protected Health Information falls under an exception to the Patient Right of Access rules discussed above.

(b) If an Inmate of a Correctional Institution makes a Request, Premier is acting under the direction of a Correctional Institution, and the Inmate's Request would jeopardize the health, safety, security, custody, or rehabilitation of the Inmates or the safety of any officer, employee or other person at the institution.

(c) If Premier obtained the requested Protected Health Information from someone other than a Health Care Provider under a *promise of confidentiality* and such access would be reasonably likely to reveal the source of the information.

v. Grounds for Denial of Access With Opportunity for Review of Denial in Certain Cases. Premier may deny a Request in any of the following three circumstances, *provided* that Premier affords the patient with a right to have the denial reviewed by a licensed health care professional designated by Premier who has not participated in the original decision to deny access:

(a) A licensed health care professional has determined that providing such access is reasonably likely to endanger the life or physical safety of the patient or another person.

(b) The requested Protected Health Information contains a reference to another person (and such other person is not a Health Care Provider) and a licensed health care professional has determined that providing the access requested is reasonably likely to cause substantial harm to such other person.

(c) The Request is made by the patient's Personal Representative and a licensed health care professional has determined that providing the requested access is reasonably likely to cause substantial harm to the patient or another person.

vi. Procedures in Case of Denial. If Premier denies access (in whole or in part) under Section K.5.iv or Section K.5.v, Premier will, to the extent possible, give the patient access to any other Protected Health Information requested after excluding the Protected Health Information to which Premier had grounds to deny access. If Premier denies the Request because it does not maintain the requested Protected Health Information, Premier will inform the patient where to direct the Request, if known.

vii. Notice of Denial Form. Premier will prepare a Denial Form containing: (i) a space to be filled in by a licensed Health Care Provider for the "reason for denial of access to Protected Health Information"; (ii) complaint procedures for patient to be directed to Premier; (iii) complaint procedures for patient to be directed to the Secretary of HHS; (iv) information describing the patient's right to a review of the denial of access and how the patient may exercise this right; and (v) contact information (name, telephone number) for the Reviewing Official for Denials of Access to Protected Health Information Requests. If Premier denies a Request, in whole or in part, Premier will furnish the patient with the completed Denial Form.

6. Review Procedures.

i. Appointment of Reviewing Official. The appropriate Privacy Officer will be responsible for appointing on a case-by-case basis a representative of Premier to serve as the Reviewing Official for Denials of Access to Health Information Requests. The Reviewing Official will be a licensed health care professional and will not have participated in the original decision to deny access.

ii. Referral to Reviewing Official. Premier will promptly refer a Request for review to the Reviewing Official.

iii. Determination by Reviewing Official. Within a reasonable time of receiving the Request for review, the Reviewing Official must determine whether or not to deny the requested access based on the standards set forth in Section K.5.v, and provide the patient with written notice of the Reviewing Official's decision. The Reviewing Official's decision will be final.

7. Fees.

i. Categories of Fees. Premier may charge the patient the following reasonable, cost-based fees associated with obtaining access to Protected Health Information subject to any state law limitations on fees:

(a) Labor for Copying: Labor fees for copying the Protected Health Information requested by the patient, subject to any state law limitations on fees.

(b) Supplies: Supplies for creating the paper copy, or the cost of Electronic Media if the patient requests that the electronic copy of the Protected Health Information be provided on portable media.

(c) Mailing: Fees may include copying costs and the cost of postage.

ii. No Handling Fees. Premier will not charge any fees for retrieving or handling the Protected Health Information or for processing the Request.

iii. Preparation of Summary. Premier may charge a reasonable per hour fee for the preparation of an explanation or summary of the Protected Health Information that Premier provides to a patient, if: (i) the patient requests such explanation and (ii) the patient agrees to be charged such fee in advance of the preparation if authorized by the appropriate Privacy Officer.

iv. Limitation of Fees for Access to Electronic Health Record in Electronic Form. Any fee that Premier imposes for providing an individual with a copy of Protected Health Information contained in an Electronic Health Record (or a summary or explanation of such information) if such copy (or summary or explanation) is in an electronic form must not be greater than Premier's labor costs in responding to the request for the copy (or summary or explanation).

8. Documentation. Premier will retain in the requesting patient's medical record, access request and denial forms and other written documentation related to access to Protected Health Information for the longer of six years from the date that the documentation was created and the last effective date of the document.

L. Amendment of Protected Health Information.

1. General Procedure. Premier recognizes the right of a patient to request that Premier amend Protected Health Information about a patient maintained in its Designated Record Set for as long as the Protected Health Information is maintained in Premier's Designated Record Set.

2. Request for Amendment. A patient must make a request for an amendment in writing. All requests must be submitted on Premier's Request for Amendment Form and provide a reason to support the requested amendment. All requests will be directed to the appropriate Privacy Officer. If a patient makes an oral request for amendment, the appropriate

Privacy Officer will inform him or her that such requests must be made in writing. Premier may refuse an oral request for amendment on the basis that such request is oral and not written.

3. Action on the Request for Amendment.

i. Timing of Response. Premier will act on a patient's request no later than 60 days after its receipt of the request.

ii. Extension of Time Frame. Premier may extend the time for action by no more than 30 days provided that it provides the patient with a written statement (within the 60-day period described above) of the reasons for the delay and the date by which Premier will complete its action on the request. Premier may have only one such 30-day extension.

iii. Determination Procedure. Premier may accept or deny the requested amendment. Determinations of whether to accept or deny the request for the amendment will be made by Premier's Medical Records Department following a review of the relevant record and Designated Record Set, consultation with the treating physician, evaluation of the patient's request, and to the extent appropriate, other health professionals familiar with the patient's course of treatment.

iv. Procedures if Amendment Accepted. If Premier accepts the Amendment:

(a) Premier will make the appropriate amendment to the Protected Health Information or record that is the subject of the request for amendment by, at a minimum, identifying the records in the Designated Record Set that are affected by the amendment and appending or otherwise providing a link to the location of the amendment.

(b) Premier will timely inform the patient in writing that the amendment has been accepted and obtain the patient's identification of and agreement to have Premier notify the relevant persons with which the amendment needs to be shared as provided below.

(c) Premier will make reasonable efforts to inform and provide the amendment within a reasonable time to:

(a) Persons identified by the patient as having received Protected Health Information about the patient and requiring the amendment; and

(b) Persons, including Business Associates of Premier, that Premier knows have the Protected Health Information that is the subject of the amendment and that may have relied, or could foreseeably rely, on such information to the detriment of the patient.

v. Procedures if Amendment Denied. If Premier denies the amendment in whole or in part:

(a) Notice of Denial. Premier will provide the patient who requested the amendment with a written denial within 60 days after receipt of the request for amendment. The denial will use plain language and contain:

(a) The basis for the denial (*i.e.*, the Protected Health Information or record that is the subject of the request (a) was not created by Premier, unless the patient provided a reasonable basis to believe that the originator of Protected Health Information is no longer available to act on the requested amendment; (b) is not part of the Designated Record Set; (c) would not be available for inspection under PH-4.K; or (d) is accurate and complete).

(b) A statement of the patient's right to submit a written statement disagreeing with the denial and how the patient may file such a statement;

(c) A statement that, if the patient does not submit a statement of disagreement, the patient may request that Premier provide the patient's request for amendment and the denial with any future disclosures of the Protected Health Information that is the subject of the amendment; and

(d) A description of how the patient may complain to Premier, or to the Director, Office for Civil Rights of HHS. The description must include the name and telephone number of Premier's Privacy Office.

(b) Written Statement of Disagreement. Premier will permit the patient to submit to Premier a written statement disagreeing with the denial of all or part of a requested amendment and the basis of such disagreement. Premier may reasonably limit the length of a statement of disagreement.

(c) Written Rebuttal to Statement. Premier will prepare a written rebuttal to the patient's statement of disagreement. Whenever such a rebuttal is prepared, Premier will provide a copy to the patient who submitted the statement of disagreement.

(d) Identification of Disputed Record or Information. Premier will, as appropriate, identify the record or Protected Health Information in the Designated Record Set that is the subject of the disputed amendment and append or otherwise link the patient's request for an amendment, Premier's denial of the request, the patient's statement of disagreement, if any, and Premier's rebuttal, if any, to the Designated Record Set.

(e) Future disclosures.

(a) If a statement of disagreement has been submitted by the patient, Premier will include the material appended, or at the election of Premier, an accurate summary of any such information, with any subsequent disclosure of the Protected Health Information to which the disagreement relates.

(b) If the patient has not submitted a written statement of disagreement, Premier must include the patient's request for amendment and its denial, or an accurate summary of such information, with any subsequent disclosure of the Protected Health Information only if the patient has requested such action in accordance with these Policies.

(c) When a subsequent disclosure is made as part of a HIPAA standard transaction under 45 C.F.R. Part 162 that does not permit the additional material to be included in the transaction, Premier may separately disclose (or transmit electronically) the material required to the recipient of the standard transaction.

vi. Notice from Another Covered Entity. If Premier is informed by another Covered Entity of an amendment to a patient's Protected Health Information, Premier will amend the Protected Health Information in the Designated Record Sets as provided by these Policies.

vii. Documentation. Premier will retain all documentation associated with requests for amendments (and the associated determinations) for the longer of: six years from the date of its creation, or the last effective date of the relevant documents. All such documentation will be maintained by appropriate Privacy Office and in the patient's record.

M. Accounting of Non-Routine Disclosures of Protected Health Information.

1. General Policy. Upon the patient's or the patient's Personal Representative's request, Premier will provide the requestor with an accounting of all Accountable Disclosures (as defined in PH-3.E) in accordance with this policy.

2. Person Who May Request an Accounting. The patient or his or her Personal Representative may request an accounting of all Accountable Disclosures of the patient's Protected Health Information. Any patient who contacts Premier to obtain an accounting will be immediately forwarded Premier's Request for Accounting Form. The patient will be encouraged to fill out the Request for Accounting Form and submit it to the appropriate Privacy Office for processing, but Premier may accept any other written request from the patient.

3. Content of Accountable Disclosures.

i. General Rule. Unless Section M.3.ii of this policy applies, Premier *must* provide the patient with a written accounting that includes all of the following with respect to each Accountable Disclosure that was made by Premier or any of its Business Associates during the accounting period (six years from the date of the patient's request, unless the patient requests a shorter time period):

- (a) The date of the disclosure;
- (b) The name of the entity or person who received the Protected Health Information and, if known, the address of such entity or person;
- (c) A brief description of the Protected Health Information disclosed; and
- (d) One of the following, as applicable:
 - (a) A brief statement of the purpose of the disclosure that reasonably informs the patient of the basis for the disclosure; or
 - (b) A copy of a written request (if any) from the Secretary of HHS ("Secretary's Request") to investigate or determine Premier's compliance with the Privacy Standards (if such Secretary's Request relates to Accountable Disclosures during the accounting period); or
 - (c) A copy a written request (if any) obtained in accordance with PH-4.D if such written request relates to Accountable Disclosures during the accounting period.

ii. Multiple Disclosures Exception. If, during the accounting period (6 years from the date of request, unless the patient requests an accounting for a shorter time period), Premier has made multiple disclosures of Protected Health Information to the same person or entity for a single purpose pursuant to (i) PH-4.D or (b) a Secretary's Request, the written accounting may, with respect to such multiple disclosures, contain the following:

- (a) The information listed above in Section M.3.i with respect to the first disclosure during the accounting period;
- (b) The frequency, periodicity, or number of the disclosures made during the accounting period; and
- (c) The date of the last such disclosure during the accounting period.

4. Timing of Response to Patient's Request for Accounting. Within 60 days after Premier's receipt of a written request for an accounting, Premier must provide the patient one of the following:

- i. A written accounting as described above; or
- ii. If Premier is unable to provide the written accounting within 60 days of Premier's receipt of the patient's written request, then a written statement of the reasons for the delay and the date by which Premier will provide the accounting (which under no circumstance may be later than ninety (90) days from the date of Premier's receipt of the patient's initial written request).

5. Fee for Accounting.

6. Retention of Accounting. Premier will retain each written accounting that it creates in accordance with this policy and each written response it provides to a patient in connection therewith for a period of six years from the date that the written accounting or other written response, as applicable, is created. In addition, Premier will retain each written request for an accounting it receives and any documentation it creates pursuant to this policy for a period of six years from the date such written request is received or such documentation is created, as applicable. All requests will be maintained by the appropriate Privacy Office.

7. Responsible Personnel.

- i. Receiving Requests. The following personnel are responsible for receiving requests for an accounting: Privacy Officer and /or their designee, in coordination with Premier Legal Department.
- ii. Processing Requests. The following personnel are responsible for processing requests for an accounting in accordance with this policy: Privacy Officer and /or their designee, in coordination with Premier Legal Department.

N. Patient's Right to Request Restrictions on Certain Uses and Disclosures.

1. General Policy. Premier recognizes the right of a patient to request that Premier restrict its:

- i. Uses or disclosures of the patient's Protected Health Information to carry out Treatment, Payment or Health Care Operations;
- ii. Disclosures of the patient's Protected Health Information to the patient's Family Member, other relative, close personal friend, or any other person identified by the patient if the Protected Health Information is directly relevant to such person's involvement with the patient's care (or Payment related to the patient's care);
- iii. Disclosures of the patient's Protected Health Information to notify, or assist in the notification of (including identifying or locating), the patient's Family

Member, Personal Representative or other person responsible for the patient's care of the patient's location, general condition or death; and

iv. Disclosures to public or private entities authorized to assist in disaster relief efforts, for the purpose of coordinating with such entities the disclosures of the patient's location, general condition or death to the patient's Family Member, Personal Representative or other person responsible for the patient's care.

2. However, except for required accommodations described in Section N.4 below, because Premier is not required to agree to such a restriction, Premier will evaluate such requests in accordance with this policy.

3. Requests to Restrict Uses and Disclosures of Protected Health Information.

i. Premier may consider a patient's request that Premier restrict its uses and disclosures described in Section N.1 above.

ii. Because Premier is not required (except as set forth in Section N.4) to agree to a restriction requested by a patient, however, it will only consider the addition of restrictions on disclosure in limited circumstances and as determined on a case-by-case basis. In all cases, no restrictions will be accepted by Premier without prior consultation among the patient, patient's physician and the appropriate Privacy Officer.

4. Required Accommodations. A Covered Entity is required to comply with the patient's request for a restriction if:

i. Except as otherwise Required By Law, the disclosure is to a Health Plan for purposes of carrying out Payment or Health Care Operations (and is not for the purposes of carrying out Treatment); and

ii. The Protected Health Information pertains solely to a health care item or service for which the Health Care Provider involved has been paid out of pocket in full.

5. Exceptions to Restrictions. If Premier has agreed to restrict the use or disclosure of Protected Health Information pursuant to Section N.3 above, Premier may use or disclose the restricted Protected Health Information in violation of such restriction if:

i. The patient who requested the restriction is in need of emergency Treatment and the restricted Protected Health Information is needed to provide the emergency Treatment. Premier may use the restricted Protected Health Information, or may disclose such information to a Health Care Provider, to provide such Treatment to the patient, provided that Premier must request that such Health Care Provider not further use or disclose such information;

ii. Such use or disclosure is required to be disclosed to the Director, Office for Civil Rights of HHS in order to investigate or determine Premier's compliance with the Privacy Standards; or

iii. Such use or disclosure is permitted or required under PH-4.D.

6. Grounds For Terminating a Restriction. Premier may terminate any restriction to its use or disclosure of such Protected Health Information, if one of the following grounds for termination exists:

i. the patient agrees to or requests the termination in writing;

ii. the patient orally agrees to the termination and the oral agreement is documented; or

iii. (a) the restriction is not required under Section N.5 of this Policy,
(b) Premier informs the patient that it is terminating its agreement to a restriction and
(c) the termination is only effective with respect to Protected Health Information about the patient created or received after Premier has so informed the patient.

7. Documentation. Premier will document any restriction in the patient's medical record and such restriction will be highlighted on the cover of the patient's chart in the same manner as patient's allergies to medication. Premier will maintain such documentation for six years from the date when the restriction was last in effect.